

ENGLISH FOR PEDIATRICIANS

Pediatrics English

Hear the child. Include the caregiver.
Explain the plan.

Phil Stilwell

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Eight lessons. Eleven extended conversations.

INSIDE THE BOOK

192 vocabulary entries / 128 reusable phrases
220 extended-dialogue turns / 11 transfer exchanges
Word banks and explained answer keys

Upper-intermediate to advanced (B2-C1). For pediatricians, pediatric trainees, and physicians coordinating care for children and adolescents.

2026 edition

Eight scenarios. Eight skills.

Eight pediatric encounters practice multiple perspectives, chart explanations, screening uncertainty, respectful disagreement, precise units, confidentiality, school coordination, and time-critical updates.

01 Hearing the child and caregiver

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A plan must reach the people who use it | Translate a clinician-approved care plan into clear responsibilities and authorized communication.

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One visit can have more than one voice.

Pediatric conversations often include a child, caregivers, schools, and several clinicians. Attribute each account, speak directly to the child when appropriate, and make the next step understandable to the people who will carry it out.

Hear the child as well as the caregiver

Accounts may differ without either person being dishonest. Ask about setting, timing, and meaning, and preserve who supplied each detail.

"You notice it at school; your caregiver notices it after dinner. Let us keep both accounts."

Translate without turning a chart into a grade

Explain what a measurement or screening result represents. Avoid calling a child a failure, a percentile a target, or a screening concern a confirmed diagnosis.

"The result tells us what needs further assessment, not everything about your child."

Verify medicine language

Distinguish amount of drug, liquid volume, concentration, and the measuring device. Use the actual verified instructions; this book does not supply doses.

"Milligrams and milliliters describe different things. We need the actual label and instructions."

Explain privacy before promising it

Confidentiality, access to records, caregiver authority, and safety duties vary by setting and jurisdiction. Explain the actual limits and communication channels.

"Let us discuss what is private, who can access the record, and the relevant safety exceptions."

PROFESSIONAL BOUNDARIES

All people, records, figures, and encounters are fictional. This is professional English practice, not medical advice, clinical training, a treatment protocol, or certification. Use current evidence, authorized supervision where required, local law, and your actual clinical and emergency procedures. Do not use an exercise to decide that symptoms can safely wait, select treatment, or calculate a dose. Communication about privacy, consent, and professional duties depends on jurisdiction and setting. Use qualified language support and protect real patient information.

Two accounts of the same week

A Read the situation

Reconcile different symptom accounts without deciding that one speaker is unreliable.

Ten-year-old Leo says stomach discomfort happens before school. His caregiver reports seeing it after dinner. Dr Park asks Leo for his account first, then clarifies each setting and timing. No cause, examination finding, or school-avoidance diagnosis is established. The two accounts may describe different episodes.

PEOPLE / Dr Park: Pediatrician | Leo: Child patient

DO NOT TURN A DIFFERENCE INTO AN ACCUSATION

A child may describe sensations differently from an adult. Distinguish what each person experienced or observed before attempting to reconcile the accounts.

Briefing checks

Circle one answer in each item. Use the briefing.

1. When does Leo report discomfort?

- A Only during examination
- C Before school

- B Never
- D Only after dinner

2. What does the caregiver observe?

- A Every episode at school
- C Discomfort after dinner

- B A completed diagnosis
- D A proven cause

3. What is not established?

- A The cause
- C Leo's report

- B Two accounts
- D A need to clarify timing

[Answers and explanations: page 85.](#)

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

presenting concern

Main issue bringing the child or family to the consultation.

IN USE clarify the presenting concern

caregiver account

Information supplied by a person caring for the child.

IN USE attribute the caregiver account

child account

Information described directly by the child.

IN USE invite the child account

symptom onset

Time when a symptom first began.

IN USE clarify symptom onset

episode duration

Length of an individual occurrence.

IN USE establish episode duration

symptom frequency

How often a symptom occurs.

IN USE describe symptom frequency

associated symptom

Additional symptom occurring with the main concern.

IN USE ask about associated symptoms

functional impact

Effect on everyday activities and participation.

IN USE assess functional impact

school attendance

Pattern of being present at school.

IN USE clarify school attendance

sleep pattern

Usual timing and quality of sleep.

IN USE describe the sleep pattern

appetite change

Difference from the person's usual desire to eat.

IN USE report an appetite change

baseline behavior

Usual behavior used for comparison.

IN USE establish baseline behavior

Precision check

Before school and after dinner may describe different episodes. Neither phrase proves a cause or a motive.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

direct observation

Information personally witnessed by the speaker.

IN USE distinguish direct observation

recalled event

Event described from memory rather than current observation.

IN USE clarify a recalled event

contextual difference

Difference in circumstances surrounding two accounts.

IN USE explore contextual differences

developmental language

Wording adapted to the child's communication development.

IN USE use developmental language

rapport

Working relationship supporting comfortable and accurate communication.

IN USE build rapport

summary check

Confirmation that a summary reflects the speaker's account.

IN USE perform a summary check

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Do not diagnose anxiety, deception, or school avoidance from the timing alone. Preserve the child and caregiver accounts as separate sources.

Phrases you can actually use

C Notice the language / 1 of 2

INVITE THE CHILD

"Leo, tell me what happens in your own words."

SEPARATE EPISODES

"Those may be two different episodes."

AVOID ACCUSATION

"The accounts differ; that does not tell us why."

CHECK MEANING

"What does that word mean when you use it?"

CLARIFY TIMING

"Is that when it starts or when someone notices it?"

ASK ABOUT IMPACT

"What becomes difficult when it happens?"

KEEP ATTRIBUTION

"Your caregiver notices it after dinner."

SUMMARIZE

"Tell me which part of my summary needs correcting."

Reports versus observes

Reports attributes an account; observes specifies something personally witnessed.

Starts versus is noticed

Separates onset from the time another person becomes aware of a symptom.

May describe

Keeps an explanation possible without asserting that it is established.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

MAKE SPACE

"We can pause before moving to the next point."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

EXPLAIN A TERM

"In everyday language, that means the following."

CHECK YOUR EXPLANATION

"Let me explain that another way."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which summary preserves the sources?

- A** The caregiver knows every episode.
- B** The timing proves school avoidance.
- C** Leo reports morning episodes; his caregiver notices evening episodes.
- D** Leo must be mistaken.

2. Which question clarifies duration?

- A** How long does one episode last?
- B** What color is the waiting room?
- C** Who is definitely wrong?
- D** Why are you avoiding school?

3. Which conclusion exceeds the facts?

- A** Two accounts are available.
- B** Leo has a school-avoidance diagnosis.
- C** Leo describes discomfort.
- D** Timing needs clarification.

4. What is a summary check for?

- A** Testing the child's intelligence
- B** Forcing agreement
- C** Replacing an examination
- D** Correcting the clinician's understanding

[Answers and explanations: page 85.](#)

Two accounts of the same week

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

recalled event / summary check / contextual difference / Direct observation / functional impact / caregiver account / developmental language / child account / episode duration / symptom frequency

- 01 **Dr Park:** Leo, I would like to hear your account first. What happens before school when your stomach feels uncomfortable?
- 02 **Leo:** It feels tight while I am getting ready, but I do not know the exact word for it.
- 03 **Dr Park:** That description helps. Your ¹ _____ does not need a medical label before I can listen carefully.
- 04 **Leo:** My caregiver said it happens after dinner. I thought that made my answer sound wrong.
- 05 **Dr Park:** The ² _____ may describe a different episode. We should clarify both rather than choose a winner.
- 06 **Leo:** Sometimes I do notice it after dinner too, but that was not the time I was thinking about.
- 07 **Dr Park:** Let us separate ³ _____ from how long each episode lasts. How often is different from how long.
- 08 **Leo:** I understand. I can remember some mornings, but I have not counted every time it happened.
- 09 **Dr Park:** Then we will preserve that uncertainty. A ⁴ _____ is not a complete record of every episode.
- 10 **Leo:** It is difficult to explain because sometimes I carry on doing things even when I notice it.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 10.

- 11 **Dr Park:** That is useful when discussing ⁵ _____. Tell me what becomes harder rather than guessing what I expect.
- 12 **Leo:** I worry that mentioning school will make everyone think I am trying not to go.
- 13 **Dr Park:** The timing alone does not establish that. A ⁶ _____ needs exploration, not an accusation about your intentions.
- 14 **Leo:** Could you ask what I mean when I use a word instead of changing it straight away?
- 15 **Dr Park:** Yes. Using suitable ⁷ _____ means checking your meaning, not replacing it with something you did not say.
- 16 **Leo:** My caregiver sees me holding my stomach at dinner, but cannot see every feeling I have at school.
- 17 **Dr Park:** Exactly. ⁸ _____ covers what someone witnesses, not every sensation you experience.
- 18 **Leo:** So you will keep the two timings separate until we understand whether they describe the same pattern?
- 19 **Dr Park:** Yes. We will clarify ⁹ _____, timing, and other details without assigning a cause from this conversation alone.
- 20 **Leo:** Then a ¹⁰ _____ lets me correct what you heard before it becomes the record.

ANSWERS / EXPLANATIONS: PAGE 86

1. child account | 2. caregiver account | 3. symptom frequency | 4. recalled event | 5. functional impact | 6. contextual difference | 7. developmental language | 8. Direct observation | 9. episode duration | 10. summary check

A different observer

F Say it

A teacher reports seeing discomfort at lunchtime. Leo has not yet discussed that episode with the clinician. The cause remains unknown.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

unknown / account / discussed / lunchtime

Teacher: "I noticed the discomfort at ¹ _____."

Clinician: "That is the teacher's ² _____."

Leo: "We have not yet ³ _____ that episode."

Clinician: "The cause remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 86

1. lunchtime | 2. account | 3. discussed | 4. unknown

Rehearse with a fixed script

1. Read the corrected conversation. Give each source its own attribution.
2. Swap roles. Contrast how often with how long without inventing a diagnosis.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A percentile is not a school grade

A Read the situation

Explain a comparison measure and its limitations without labeling a child by a number.

Caregiver Ana sees the 25th percentile on a growth chart and worries that her child has scored only 25 out of 100. Dr Bell explains that a percentile compares a particular measurement with a reference population; it is not a grade or a target. The full growth trajectory, measurement quality, and clinical context still require review. No growth disorder is diagnosed in the case.

PEOPLE / Ana: Caregiver | Dr Bell: Pediatrician

FIND THE MISTAKEN COMPARISON FIRST

Before giving more numbers, ask what the existing number means to the caregiver. Correct the interpretation without treating the concern as foolish.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Ana mistake the percentile for?

- A A score out of 100
- C A medicine volume

- B A date
- D A confirmed diagnosis

2. What still needs review?

- A Nothing beyond one point
- C Only the chart color

- B The trajectory and clinical context
- D A predetermined target of 100

3. What is not diagnosed?

- A Ana's concern
- C A misunderstanding

- B A chart entry
- D A growth disorder

Answers and explanations: page 87.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

growth trajectory

Pattern of growth measurements over time.

IN USE review the growth trajectory

percentile

Position of a measurement relative to a reference distribution.

IN USE explain a percentile

reference population

Group providing the comparison distribution for a measure.

IN USE identify the reference population

serial measurements

Measurements collected at different times for comparison.

IN USE review serial measurements

growth velocity

Rate of change in growth over time.

IN USE assess growth velocity

weight-for-age

Weight compared with an age-specific reference.

IN USE plot weight-for-age

length-for-age

Recumbent length compared with an age-specific reference.

IN USE review length-for-age

stature-for-age

Standing height compared with an age-specific reference.

IN USE assess stature-for-age

head circumference

Measurement around the head using the appropriate method.

IN USE record head circumference

body mass index

Weight relative to squared height, interpreted appropriately for age.

IN USE explain body mass index

measurement technique

Method used to obtain a physical measurement.

IN USE verify measurement technique

plotting error

Incorrect placement or entry of data on a chart.

IN USE check for a plotting error

Precision check

The 25th percentile is not 25 percent healthy, 25 percent grown, or a target to improve toward 100.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

centile crossing

Movement across reference percentile lines over time.

IN USE review centile crossing

growth faltering

Growth pattern raising concern about inadequate progress in context.

IN USE assess suspected growth faltering

familial pattern

Pattern related to relevant family characteristics.

IN USE consider the familial pattern

nutritional history

Account of intake and related nutrition circumstances.

IN USE obtain a nutritional history

corrected age

Age adjusted for prematurity when appropriate to the assessment.

IN USE clarify corrected age

longitudinal assessment

Evaluation using information collected across time.

IN USE conduct a longitudinal assessment

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

A single plotted point cannot establish the whole growth assessment. Chart selection, measurement quality, timing, and context matter.

Phrases you can actually use

C Notice the language / 1 of 2

CHECK THE INTERPRETATION

"What did the number suggest to you?"

NAME THE COMPARISON

"It compares this measurement with a reference group."

CHECK THE DATA

"We should verify how this measurement was taken."

LIMIT THE CONCLUSION

"This point alone does not establish a growth disorder."

REJECT THE GRADE ANALOGY

"This is not a score out of 100."

SHIFT TO THE PATTERN

"Let us look at the measurements over time."

AVOID A TARGET

"A higher percentile is not automatically a better result."

EXPLAIN THE REVIEW

"The pattern and the child's circumstances belong together."

Relative to

Identifies a comparison group instead of implying an absolute health score.

Over time

Moves attention from an isolated point to a trajectory.

Not automatically

Rejects a simplistic inference without declaring every measurement unimportant.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which explanation is accurate?

- A** Every child should reach the 100th.
- B** A percentile locates a measurement within a reference distribution.
- C** It is the percentage of health achieved.
- D** It is the percentage of adult size reached.

2. What does growth velocity describe?

- A** A single measurement's color
- B** The percentage of healthy organs
- C** Rate of growth over time
- D** The child's running speed

3. Which check improves interpretation?

- A** Ignore all earlier values.
- B** Assume a disorder from one point.
- C** Use a guessed age.
- D** Verify measurement technique and prior points.

4. Which conclusion is unsupported?

- A** Ana misunderstands the number.
- B** A chart entry exists.
- C** Context requires review.
- D** This child has a diagnosed growth disorder.

Answers and explanations: page 87.

A percentile is not a school grade

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

plotting error / Growth velocity / percentile / serial measurements / measurement technique / growth trajectory / centile crossing / nutritional history / longitudinal assessment / reference population

- 01 **Ana:** The chart says 25th percentile. I thought that meant my child had scored only 25 out of 100.
- 02 **Dr Bell:** A ¹ _____ is not a school grade. It tells us where a particular measurement sits in a comparison group.
- 03 **Ana:** So it does not mean that my child is only a quarter as healthy as expected?
- 04 **Dr Bell:** Correct. The ² _____ provides the comparison; it does not turn a higher number into an automatic goal.
- 05 **Ana:** Then what do you look at when deciding whether the growth pattern needs attention?
- 06 **Dr Bell:** We review the ³ _____, the quality of the measurements, and the child's wider clinical circumstances.
- 07 **Ana:** I have earlier measurements from another clinic, but I am not sure they used the same method.
- 08 **Dr Bell:** Those ⁴ _____ may help, provided we check their timing, method, and comparability rather than assume everything matches.
- 09 **Ana:** Would a mistake in entering the height make the chart look different even if nothing changed?
- 10 **Dr Bell:** A ⁵ _____ can affect the display. That is one reason to verify the underlying data.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 19.

- 11 **Ana:** I also heard the phrase growth velocity. Is that another word for the percentile?
- 12 **Dr Bell:** No. ⁶ _____ describes the rate of growth over time, not the comparison position at one visit.
- 13 **Ana:** So two children at the same percentile today might still have different histories?
- 14 **Dr Bell:** Yes. A ⁷ _____ considers the sequence, not merely whether two points happen to share a label.
- 15 **Ana:** Does that mean I should stop paying attention to the measurements altogether?
- 16 **Dr Bell:** Not at all. We check ⁸ _____ and context because the measurements matter; we simply avoid treating them as grades.
- 17 **Ana:** What other information would help explain the pattern when you review it?
- 18 **Dr Bell:** Relevant history can include a ⁹ _____ and family information, alongside the actual examination and other clinical details.
- 19 **Ana:** I understand. The number alone does not tell us that a particular disorder is present.
- 20 **Dr Bell:** Exactly. Even ¹⁰ _____ needs interpretation. We will explain the reviewed pattern without making a diagnosis from this single point.

ANSWERS / EXPLANATIONS: PAGE 88

1. percentile | 2. reference population | 3. growth trajectory | 4. serial measurements | 5. plotting error | 6. Growth velocity | 7. longitudinal assessment | 8. measurement technique | 9. nutritional history | 10. centile crossing

A copied height needs checking

F Say it

A previous height was copied incorrectly into the chart. The original measurement is being verified before the trajectory is interpreted.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

disorder / measurement / interpreted / incorrect

Caregiver: "The copied height may be ¹ _____."

Clinician: "We are checking the original ² _____."

Caregiver: "The trajectory is not yet ³ _____."

Clinician: "An entry error is not itself a growth ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 88

1. incorrect | 2. measurement | 3. interpreted | 4. disorder

Rehearse with a fixed script

1. Read the corrected chart explanation, contrasting comparison, rate, and grade.
2. Swap roles. Explain the 25th percentile without setting a numerical growth target.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A concern is a reason to assess

A Read the situation

03

Explain the difference between surveillance, screening, evaluation, and diagnosis.

A developmental questionnaire raises a communication concern for toddler Nia. Her caregiver Ben worries that the form has already diagnosed a lifelong condition. Dr Shah explains the role and limits of screening, asks about abilities across settings and languages, and arranges the actual assessment pathway. No specific diagnosis or referral appointment date is supplied.

PEOPLE / Ben: Caregiver | Dr Shah: Pediatrician

USE CONCERN WITHOUT TURNING IT INTO A LABEL

Acknowledge the caregiver's uncertainty. Describe the next step concretely and avoid promising either a diagnosis or that the concern will disappear.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What raises the concern?

- A** A developmental questionnaire
- C** An employer report

- B** A confirmed diagnostic evaluation
- D** A treatment trial

2. What does Ben fear?

- A** That the form is blank
- C** That an appointment is already complete

- B** That no one read it
- D** That a lifelong diagnosis is already settled

3. What is not supplied?

- A** Nia's name
- C** An assessment pathway

- B** A communication concern
- D** A specific diagnosis or appointment date

Answers and explanations: [page 89](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

developmental surveillance

Ongoing attention to a child's development across encounters.

IN USE conduct developmental surveillance

developmental screening

Structured process identifying possible developmental concerns.

IN USE explain developmental screening

diagnostic evaluation

Detailed assessment intended to establish or clarify a diagnosis.

IN USE arrange a diagnostic evaluation

developmental milestone

Skill commonly emerging within a developmental period.

IN USE discuss developmental milestones

expressive language

Ability to communicate ideas using words or other expressive means.

IN USE describe expressive language

receptive language

Ability to understand communicated language.

IN USE assess receptive language

gross motor skill

Skill involving larger body movements and muscle groups.

IN USE review gross motor skills

fine motor skill

Skill involving smaller coordinated movements.

IN USE describe fine motor skills

social communication

Communication used in interaction with other people.

IN USE explore social communication

adaptive skill

Practical ability used in everyday functioning.

IN USE assess adaptive skills

caregiver questionnaire

Structured form completed by someone caring for the child.

IN USE review a caregiver questionnaire

screen-positive result

Screening result indicating that further assessment is warranted.

IN USE explain a screen-positive result

Precision check

A screen-positive result identifies a need for further assessment; it is not a diagnosis. Multilingual exposure should be described accurately, not used as an automatic explanation for every concern.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

false-positive screen

Screening concern not confirmed by the relevant further assessment.

IN USE explain a possible false-positive screen

early intervention

Support addressing developmental needs early in childhood.

IN USE discuss early-intervention services

hearing assessment

Evaluation of hearing relevant to communication and other concerns.

IN USE arrange a hearing assessment

multilingual exposure

Experience with more than one language.

IN USE describe multilingual exposure

skill regression

Loss of a previously acquired ability.

IN USE report skill regression

referral status

Current stage of a referral's receipt and processing.

IN USE check referral status

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Referral sent, referral received, and appointment booked are separate states. Do not promise that an assessment has occurred because a form was completed.

Phrases you can actually use

C Notice the language / 1 of 2

03

NAME THE ROLE

"This questionnaire identifies concerns that need a closer look."

SEPARATE THE CONCLUSION

"It does not establish a diagnosis by itself."

ASK ACROSS SETTINGS

"What do you notice at home and in other settings?"

INCLUDE LANGUAGES

"Tell me which languages Nia hears and uses."

CLARIFY A CHANGE

"Is this a skill not yet acquired or one that was lost?"

EXPLAIN EVALUATION

"The assessment looks at more information than this form."

NAME THE NEXT STEP

"We will explain the referral and how to check its status."

AVOID A FALSE PROMISE

"I cannot promise the assessment's conclusion in advance."

Not yet acquired versus lost

Distinguishes delayed acquisition from regression without assigning a cause.

Flags versus confirms

Flags identifies a concern; confirms requires the relevant diagnostic evidence.

Received versus booked

Prevents referral administration from being mistaken for a scheduled assessment.

Phrases you can actually use

C Notice the language / 2 of 2

03

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

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"I will distinguish what was reported from what was observed."

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"We have agreed on the next step, not on an outcome."

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Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement preserves the distinction?

- A** Every concern is automatically false.
- B** Screening flags a concern; evaluation investigates it.
- C** Screening proves a lifelong diagnosis.
- D** A questionnaire replaces all assessment.

2. Which question clarifies regression?

- A** Was the skill previously present and then lost?
- B** Is the form printed in blue?
- C** Will the diagnosis definitely disappear?
- D** Can we skip the history?

3. How should multilingual exposure be handled?

- A** Describe it accurately within the assessment.
- B** Assume it explains every concern.
- C** Ignore all languages but English.
- D** Treat it as a diagnosis.

4. Which referral claim is unsupported?

- A** An appointment is already booked.
- B** A pathway is being arranged.
- C** The status needs checking.
- D** No date is supplied.

Answers and explanations: [page 89](#).

A concern is a reason to assess

E Conversations / turns 1-10

03

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

early intervention / social communication / Skill regression / Receptive language / developmental surveillance / referral status / diagnostic evaluation / hearing assessment / Developmental screening / multilingual exposure

- 01 **Ben:** The questionnaire flagged a communication concern. Has it already diagnosed Nia with something that will last her whole life?
- 02 **Dr Shah:** No. ¹ _____ identifies concerns for a closer look; it does not settle a diagnosis by itself.
- 03 **Ben:** I thought the score meant the assessment had already been completed and the conclusion was final.
- 04 **Dr Shah:** A ² _____ considers more information, including history, observation, and the relevant assessments.
- 05 **Ben:** What kinds of language skills are you asking about? She understands some things she does not say.
- 06 **Dr Shah:** ³ _____ concerns understanding, while expressive language concerns what she communicates. We should distinguish those accounts.
- 07 **Ben:** She hears two languages at home and uses different words with different relatives.
- 08 **Dr Shah:** Please describe that ⁴ _____ accurately. We should not assume it explains every concern or ignore abilities in another language.
- 09 **Ben:** Some questions also asked about pointing and how she interacts with other children.
- 10 **Dr Shah:** Those can relate to ⁵ _____, which is broader than counting spoken words alone.

The conversation, continued

E Conversations / turns 11-20

03

Continue with the word bank on page 28.

- 11 **Ben:** If she has never used a particular word, is that the same as losing a skill?
- 12 **Dr Shah:** No. ⁶ _____ means an ability was present and then lost. The distinction matters in the history.
- 13 **Ben:** Could hearing be part of the assessment, even if we think she hears some sounds?
- 14 **Dr Shah:** A relevant ⁷ _____ may form part of the actual pathway. An informal impression does not replace the appropriate assessment.
- 15 **Ben:** Does getting support have to mean that every diagnostic question has already been answered?
- 16 **Dr Shah:** We can discuss appropriate ⁸ _____ and the local process while explaining what remains uncertain.
- 17 **Ben:** I would like to know whom to contact if we do not hear about the referral.
- 18 **Dr Shah:** We will explain how to check ⁹ _____, including the difference between sending a request and having a confirmed appointment.
- 19 **Ben:** That gives us a next step without pretending the questionnaire has answered everything.
- 20 **Dr Shah:** Exactly. Ongoing ¹⁰ _____ continues alongside the appropriate assessment and communication with your family.

ANSWERS / EXPLANATIONS: PAGE 90

1. Developmental screening | 2. diagnostic evaluation | 3. Receptive language | 4. multilingual exposure | 5. social communication | 6. Skill regression | 7. hearing assessment | 8. early intervention | 9. referral status | 10. developmental surveillance

Receipt is confirmed, booking is not

F Say it

03

The developmental service confirms receipt of Nia's referral. No appointment has been booked. The family has a contact for checking progress.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

receipt / contact / pending / booked

Caregiver: "The service has confirmed ¹ _____."

Clinician: "An appointment is not yet ² _____."

Caregiver: "We have a progress-check ³ _____."

Clinician: "The diagnostic conclusion remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 90

1. receipt | 2. booked | 3. contact | 4. pending

Rehearse with a fixed script

1. Read the corrected screening discussion, keeping each assessment stage distinct.
2. Swap roles. Explain the next step without predicting a diagnosis or an appointment date.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A concern deserves a specific answer

A Read the situation

Elicit the actual concern and explain a recommendation without shaming or claiming zero risk.

Caregiver Dana worries about fever after a previous vaccination and asks whether the same thing will happen again. Dr Lin asks what occurred and when, reviews the actual record, and explains that timing alone does not establish causation. The clinician will review the current recommendation and individual history. No vaccine schedule, eligibility decision, or product-specific risk estimate is supplied.

PEOPLE / Dana: Caregiver | Dr Lin: Pediatrician

SPECIFIC QUESTIONS BEAT LABELS

Ask what the caregiver observed and fears. Labels such as difficult or anti-science obscure the question and can damage the discussion.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What worries Dana?

- | | |
|---|--|
| A Fever after a previous vaccination | B A known product recall |
| C A supplied contraindication | D A confirmed repeated reaction |

2. What needs review?

- | | |
|--|--|
| A The event history and actual recommendation | B Only the caregiver's attitude |
| C A guessed schedule | D An automatic exemption |

3. What is not supplied?

- | | |
|---------------------------------|---|
| A A caregiver concern | B A previous event |
| C A clinician discussion | D A product-specific risk estimate |

Answers and explanations: page 91.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

immunization

Process of developing protection against a disease, commonly through vaccination.

IN USE discuss immunization

vaccine recommendation

Current advice about use of a particular vaccine in context.

IN USE review the vaccine recommendation

vaccination history

Record of previous vaccine administration.

IN USE verify vaccination history

adverse event

Unwanted occurrence after an intervention, not necessarily caused by it.

IN USE document an adverse event

adverse reaction

Harmful response attributed to an intervention.

IN USE distinguish an adverse reaction

temporal association

Events occurring in a time relationship without proving causation.

IN USE explain temporal association

causal relationship

Connection in which one event contributes to producing another.

IN USE assess a causal relationship

contraindication

Condition making a particular intervention inappropriate under applicable guidance.

IN USE verify a contraindication

precaution

Circumstance requiring additional consideration before an intervention.

IN USE review a relevant precaution

expected reaction

Known response discussed in the context of a specific product.

IN USE explain an expected reaction

vaccine hesitancy

Uncertainty or reluctance concerning vaccination.

IN USE explore vaccine hesitancy

informed choice

Decision made after relevant information and questions are addressed.

IN USE support informed choice

Precision check

After vaccination describes timing. Caused by vaccination requires a causal assessment. Do not dismiss the event or declare its cause without review.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

disease protection

Reduction in disease risk or consequences through an intervention.

IN USE explain disease protection

breakthrough infection

Infection occurring despite relevant vaccination or protection.

IN USE discuss breakthrough infection

catch-up schedule

Recommended sequence addressing previously missed vaccine doses.

IN USE verify a catch-up schedule

administration record

Documentation of a product given and its relevant details.

IN USE check the administration record

safety monitoring

Ongoing observation and review of potential safety issues.

IN USE explain safety monitoring

absolute risk

Probability or frequency of an outcome in a defined group and period.

IN USE communicate absolute risk

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Use current, jurisdiction-appropriate recommendations and the actual clinical history. This exercise supplies no schedule, contraindication decision, or guarantee of no reaction.

Phrases you can actually use

C Notice the language / 1 of 2

FIND THE CONCERN

"What happened after the previous vaccination?"

ACKNOWLEDGE THE EXPERIENCE

"I can understand why that experience raised a question."

REVIEW THE RECORD

"Let us check the actual administration record."

INVITE A FOCUSED QUESTION

"Which part of the recommendation remains unclear?"

CLARIFY TIMING

"How soon afterward did you notice the change?"

SEPARATE CAUSE AND TIMING

"Happening afterward does not by itself establish the cause."

AVOID ZERO-RISK LANGUAGE

"I will explain the relevant benefits and possible harms."

CLOSE WITH UNDERSTANDING

"Please tell me how you understand the next step."

After versus because of

After states sequence; because of assigns a cause.

May recur versus will recur

A possible recurrence is not a prediction that the event is certain.

Current recommendation

Keeps the discussion tied to applicable guidance rather than a remembered universal schedule.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which wording preserves causation uncertainty?

- A** Fever occurred after vaccination; its cause needs review.
- B** Vaccination definitely caused every later symptom.
- C** Timing proves causation.
- D** Nothing happened because causation is uncertain.

2. Which claim is unsupported?

- A** The actual recommendation needs checking.
- B** No reaction can ever occur.
- C** We can review the history.
- D** Your question deserves an answer.

3. What is an adverse event?

- A** A guaranteed harmless response
- B** A completed causal investigation
- C** An unwanted occurrence not necessarily caused by the intervention
- D** Proof of a contraindication in every case

4. Which question is most useful first?

- A** Why are you being difficult?
- B** Can you agree without questions?
- C** Who told you to worry?
- D** What happened and when?

Answers and explanations: [page 91](#).

A concern deserves a specific answer

E Conversations / turns 1-10

04

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

contraindication / administration record / catch-up schedule / informed choice / temporal association / causal relationship / adverse event / absolute risk / vaccine recommendation / vaccination history

- 01 **Dana:** My child had a fever after the last vaccination. Does that mean the same thing will happen again?
- 02 **Dr Lin:** Let us first review the ¹ _____ and what you noticed, including when it started and what happened afterward.
- 03 **Dana:** I appreciate that. I was worried you would think asking about it meant refusing every vaccine.
- 04 **Dr Lin:** Asking a question is not a blanket refusal. Your account helps us review the relevant ² _____ rather than dismiss it.
- 05 **Dana:** The timing seemed very close, so I assumed that was enough to prove the cause.
- 06 **Dr Lin:** A ³ _____ tells us when things happened. It does not, by itself, settle the cause.
- 07 **Dana:** So you are not saying the fever did not happen, only that its explanation needs review?
- 08 **Dr Lin:** Exactly. A ⁴ _____ is a separate question from whether you observed an event.
- 09 **Dana:** What if something in the medical history means a particular vaccine should not be given?
- 10 **Dr Lin:** We review any relevant ⁵ _____ and precautions using the actual history and current guidance.

The conversation, continued

E Conversations / turns 11-20

04

Continue with the word bank on page 37.

- 11 **Dana:** I would like to hear the benefit as well as what unwanted effects are possible.
- 12 **Dr Lin:** The ⁶ _____ needs that discussion. I should not replace it with a promise of zero risk.
- 13 **Dana:** Sometimes a percentage is hard to understand without knowing how many children it refers to.
- 14 **Dr Lin:** When relevant figures are available, ⁷ _____ needs the outcome, group, and time period made clear.
- 15 **Dana:** I also have a record from another clinic, but I am unsure whether every entry was copied.
- 16 **Dr Lin:** We should verify the ⁸ _____ rather than assume a copied list is complete.
- 17 **Dana:** Does that mean you will create a catch-up plan just from what I remember today?
- 18 **Dr Lin:** No. Any ⁹ _____ requires the verified history and applicable recommendation, not a guessed sequence.
- 19 **Dana:** That helps me ask a more specific question instead of trying to decide everything from one memory.
- 20 **Dr Lin:** Good. An ¹⁰ _____ depends on clear information and a chance to resolve the questions that matter to you.

ANSWERS / EXPLANATIONS: PAGE 92

1. vaccination history | 2. adverse event | 3. temporal association | 4. causal relationship | 5. contraindication | 6. vaccine recommendation | 7. absolute risk | 8. administration record | 9. catch-up schedule | 10. informed choice

An incomplete outside record

F Say it

An outside vaccination record contains an unclear entry. The clinic requests clarification before treating it as a confirmed administration.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

clarification / unclear / record / confirmed

Caregiver: "This entry is ¹ _____."

Clinician: "We have requested ² _____."

Caregiver: "Administration is not yet ³ _____."

Clinician: "We will use the verified ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 92

1. unclear | 2. clarification | 3. confirmed | 4. record

Rehearse with a fixed script

1. Read the corrected exchange. Distinguish timing from a causal claim.
2. Swap roles. Acknowledge the concern without shaming, prescribing a schedule, or promising zero risk.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

The label, the units, and the tool

A Read the situation

Distinguish drug amount from liquid volume and check the actual instructions without calculating a dose.

05

Caregiver Omar sees mg on a prescription and mL on a measuring device and assumes they are interchangeable. Dr Cole explains the difference between drug amount, liquid volume, and concentration. The actual bottle, prescription, and appropriate measuring tool must be checked with the prescribing or dispensing team. No numerical dose, concentration, interval, or instruction to administer medicine is provided here.

PEOPLE / Omar: Caregiver | Dr Cole: Pediatrician

TEACH-BACK CHECKS THE EXPLANATION

Invite the caregiver to show how the verified instructions would be followed. Correct a misunderstanding without treating it as evidence of carelessness.

Briefing checks

Circle one answer in each item. Use the briefing.

1. Which units does Omar confuse?

A cm and meters

B Hours and dates

C Degrees and percentiles

D mg and mL

2. What must be checked?

A Another child's instructions

B The actual bottle, prescription, and measuring tool

C A remembered spoonful

D A guessed concentration

3. What is deliberately not supplied?

A A unit misunderstanding

B A need to verify instructions

C A caregiver question

D A numerical dose

Answers and explanations: [page 93](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

milligram

Unit of mass used to express an amount of drug.

IN USE distinguish milligrams

milliliter

Unit of liquid volume, abbreviated mL.

IN USE measure in milliliters

concentration

Amount of drug contained in a stated volume or quantity.

IN USE verify the concentration

dose amount

Quantity of drug prescribed for an administration.

IN USE confirm the dose amount

dose volume

Volume of a liquid corresponding to a verified dose.

IN USE verify the dose volume

oral syringe

Measuring device designed for administering liquid medicine by mouth.

IN USE select the appropriate oral syringe

calibration mark

Mark indicating a measured quantity on a device.

IN USE read the calibration marks

prescription label

Written instructions attached to a dispensed medicine.

IN USE check the prescription label

active ingredient

Substance responsible for the medicine's intended pharmacological action.

IN USE identify the active ingredient

duplicate ingredient

Same active substance present in more than one product.

IN USE check for duplicate ingredients

administration interval

Time between prescribed administrations.

IN USE confirm the administration interval

as-needed instruction

Direction describing use when specified conditions apply.

IN USE clarify an as-needed instruction

Precision check

Milligrams measure drug mass; milliliters measure liquid volume. The relationship depends on the actual concentration, which must not be guessed.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

maximum daily amount

Highest permitted amount over the specified daily period.

IN USE verify the maximum daily amount

measuring device

Tool used to determine the required quantity.

IN USE match the measuring device

dispensing pharmacy

Pharmacy supplying the prescribed product.

IN USE contact the dispensing pharmacy

label discrepancy

Conflict between the label and another instruction or record.

IN USE resolve a label discrepancy

caregiver demonstration

Caregiver showing how an instruction is understood.

IN USE invite a caregiver demonstration

medication reconciliation

Process of verifying and aligning the actual medicine list.

IN USE complete medication reconciliation

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Use the prescribed, verified instructions and appropriate device. Household spoons are not reliable measuring tools. This exercise does not authorize dose calculation or administration.

Phrases you can actually use

C Notice the language / 1 of 2

SEPARATE THE UNITS

"Milligrams and milliliters measure different things."

USE THE REAL PRODUCT

"We need to check this bottle and its current label."

RESOLVE A CONFLICT

"Let us clarify the conflicting instructions with the responsible team."

INVITE DEMONSTRATION

"Show me how you understand the verified instruction."

NAME THE MISSING LINK

"The actual concentration connects amount and volume."

AVOID SUBSTITUTION

"Do not assume another bottle has the same concentration."

CHECK THE TOOL

"The measuring device needs to match the verified instructions."

OWN THE EXPLANATION

"Let me explain the units more clearly."

Amount versus volume

Keeps the quantity of drug distinct from the volume of liquid containing it.

This bottle

Anchors the check to the actual product rather than a remembered label.

Show me

Checks practical understanding of verified instructions, not mental arithmetic from an incomplete scenario.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which statement is correct?

- A** mg measures mass; mL measures volume.
- B** mg and mL always mean the same quantity.
- C** A spoonful fixes every dose.
- D** Concentration never matters.

2. What must not be guessed?

- A** The concentration
- B** The need for clarification
- C** Omar's stated confusion
- D** The existence of a measuring device

3. Which tool is inappropriate for precise measurement?

- A** An appropriate verified oral syringe
- B** The pharmacist-specified calibrated device
- C** A suitable labeled measuring tool
- D** An ordinary kitchen spoon

4. What is the purpose of demonstration?

- A** Test the caregiver's intelligence
- B** Invent a dose
- C** Replace the actual prescription
- D** Check understanding of verified instructions

Answers and explanations: [page 93](#).

The label, the units, and the tool

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

05

WORD + PHRASE BANK

milligram / prescription label / label discrepancy / active ingredient / dispensing pharmacy / caregiver demonstration / concentration / calibration mark / dose volume / oral syringe

- 01 **Omar:** The prescription says mg, but the measuring device says mL. I thought they meant the same amount.
- 02 **Dr Cole:** A ¹ _____ measures an amount of drug by mass. A milliliter measures liquid volume.
- 03 **Omar:** Then I cannot just copy the same number from one unit onto the other?
- 04 **Dr Cole:** Correct. The actual ² _____ links drug amount and liquid volume. We must not guess it.
- 05 **Omar:** I have an older bottle at home. Could I use its instructions if the name looks similar?
- 06 **Dr Cole:** We need this product's current ³ _____ and verified instructions, not a remembered label from a different bottle.
- 07 **Omar:** I also need to know which measuring tool to use. A kitchen spoon seems easier.
- 08 **Dr Cole:** A kitchen spoon is not a reliable measure. The appropriate ⁴ _____ or other specified device should match the verified instructions.
- 09 **Omar:** Some of the small lines on the device are difficult for me to distinguish.
- 10 **Dr Cole:** We should check the ⁵ _____ spacing and whether the device is suitable, rather than asking you to estimate between unclear lines.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 46.

05

- 11 **Omar:** If the label and another instruction disagree, which one should I guess is current?
- 12 **Dr Cole:** Do not guess. A ⁶ _____ needs clarification with the responsible prescribing or dispensing team.
- 13 **Omar:** Would the pharmacy be able to check the product it actually supplied?
- 14 **Dr Cole:** Yes, the ⁷ _____ is an appropriate contact for verifying the dispensed product and its instructions.
- 15 **Omar:** I use another over-the-counter product sometimes. Should that be part of the review too?
- 16 **Dr Cole:** Yes. Reviewing the ⁸ _____ helps identify possible duplication; similar or different brand names do not settle that question.
- 17 **Omar:** Once everything is verified, can I show you how I understand the instruction?
- 18 **Dr Cole:** A ⁹ _____ is useful. It checks our explanation and the tool, not your intelligence.
- 19 **Omar:** I can now distinguish the amount of drug from the liquid that contains it.
- 20 **Dr Cole:** Exactly. The ¹⁰ _____ must match the verified instructions. Let us check the bottle, prescription, and measuring device together before confirming what to give.

ANSWERS / EXPLANATIONS: PAGE 94

1. milligram | 2. concentration | 3. prescription label | 4. oral syringe | 5. calibration mark | 6. label discrepancy | 7. dispensing pharmacy | 8. active ingredient | 9. caregiver demonstration | 10. dose volume

Two products may share an ingredient

F Say it

A caregiver brings two products with different brand names. Their active ingredients have not yet been checked. The clinician asks the responsible team to review them.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

review / unchecked / different / duplication

Caregiver: "The brand names are ¹ _____."

Clinician: "The active ingredients remain ² _____."

Caregiver: "We should not assume there is no ³ _____."

Clinician: "The responsible team will ⁴ _____ them."

ANSWERS / EXPLANATIONS: PAGE 94

1. different | 2. unchecked | 3. duplication | 4. review

Rehearse with a fixed script

1. Read the corrected exchange. Say milligrams and milliliters in full at first mention.
2. Swap roles. Practice resolving the instruction conflict without inventing a dose or concentration.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Privacy without an impossible promise

A Read the situation

Explain private discussion, record access, and safety limits before sensitive disclosure.

Sixteen-year-old Alex asks Dr Hale whether everything discussed will remain private. Dr Hale explains that confidentiality, consent, caregiver access, portal settings, billing, and safety duties depend on the applicable rules and service. The clinician offers an appropriate private discussion and checks actual access arrangements. The case supplies no jurisdiction-specific legal conclusion.

PEOPLE / Alex: Adolescent patient | Dr Hale: Pediatrician

06

EXPLAIN LIMITS BEFORE COLLECTING THE DETAIL

Do not promise absolute secrecy and then reveal exceptions later. Make the actual privacy arrangements understandable without discouraging the adolescent from seeking care.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does Alex ask?

- | | |
|--|---|
| A Whether billing is always invisible | B Whether everything will remain private |
| C Whether every record is already deleted | D Whether a law has been supplied |

2. What does Dr Hale check?

- | | |
|--|--|
| A Actual access arrangements and applicable rules | B A universal rule from memory |
| C Only the room number | D Nothing beyond verbal reassurance |

3. What is not supplied?

- | | |
|----------------------------------|---|
| A Alex's age | B A privacy question |
| C A clinician explanation | D A jurisdiction-specific legal conclusion |

Answers and explanations: page 95.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

confidentiality

Duty to protect information within applicable rules and limits.

IN USE explain confidentiality

private consultation

Discussion held without others present when appropriate.

IN USE offer a private consultation

consent capacity

Ability to make the relevant informed decision under applicable standards.

IN USE assess consent capacity

minor consent

Rules governing a minor's authorization for particular care.

IN USE verify minor-consent rules

guardian authority

Legally recognized decision-making role for a child in context.

IN USE clarify guardian authority

proxy access

Another person's permitted access to a patient's information system.

IN USE check proxy access

portal visibility

Information viewable through a patient-access system.

IN USE verify portal visibility

billing disclosure

Information potentially revealed through payment or insurance processes.

IN USE explain possible billing disclosure

safety exception

Circumstance permitting or requiring disclosure for safety under applicable rules.

IN USE explain a safety exception

mandatory reporting

Legally required communication of specified concerns to an authority.

IN USE clarify mandatory-reporting duties

information sharing

Disclosure of relevant information to another person or service.

IN USE discuss information sharing

minimum necessary information

Limited information appropriate to a particular authorized purpose.

IN USE identify necessary information

06

Precision check

Privacy in the room does not automatically make portal, billing, or later communications private. Check the actual channels.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

preferred contact method

Communication route the person wishes the service to use.

IN USE confirm the preferred contact method

confidential contact

Communication handled through an appropriately private route.

IN USE arrange a confidential contact

record-access rule

Requirement governing who can see particular health information.

IN USE verify record-access rules

privacy limitation

Boundary on what can be kept private in the actual setting.

IN USE explain privacy limitations

safeguarding concern

Concern about safety or welfare requiring appropriate assessment or action.

IN USE address a safeguarding concern

supported disclosure

Planned sharing of information with appropriate explanation and support.

IN USE arrange supported disclosure

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Avoid universal claims about minor consent or caregiver access. Explain the applicable rules, safety obligations, and any limits before promising confidentiality.

Phrases you can actually use

C Notice the language / 1 of 2

AVOID ABSOLUTE SECRECY

"I cannot promise that every circumstance is completely private."

OFFER PRIVATE TIME

"We can arrange an appropriate conversation without your caregiver present."

DISCUSS CHANNELS

"Portal messages and billing may have different access arrangements."

ASK ABOUT CONTACT

"Which contact method is appropriate for us to use?"

EXPLAIN BEFORE DISCLOSURE

"Let us discuss the privacy limits before you share the detail."

CHECK ACCESS

"I need to verify who can see this part of the record."

EXPLAIN SAFETY

"I will explain the relevant safety duties in this setting."

SUPPORT SHARING

"When sharing is needed, I will explain the process and support you."

06

Private versus confidential

Private describes the setting; confidentiality concerns handling information under applicable duties.

May have access

Preserves uncertainty until the actual access arrangements are checked.

Before you share

Places the explanation of limits before sensitive detail is requested.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

06

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which promise is unsafe?

- A** Let us confirm the contact route.
- B** Nothing you say can ever be shared under any circumstances.
- C** We can discuss the actual limits.
- D** I will check portal access.

2. Which issue is separate from privacy in the room?

- A** The appointment font
- B** Portal and billing visibility
- C** The chair color
- D** The clinician's pen

3. Which legal statement is justified here?

- A** Every caregiver sees every record everywhere.
- B** No safety duty can apply.
- C** Applicable rules must be verified.
- D** Every minor has identical rights everywhere.

4. What should happen before sensitive disclosure?

- A** Ignore contact preferences
- B** Explain relevant privacy limits
- C** Promise secrecy without checking
- D** Ask for every detail first

Answers and explanations: [page 95](#).

Privacy without an impossible promise

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

preferred contact method / record-access rule / Minor consent / confidentiality / Mandatory reporting / proxy access / safety exception / billing disclosure / private consultation / supported disclosure

06

- 01 **Alex:** Before I say anything personal, can you promise that everything stays between us and nobody else can see it?
- 02 **Dr Hale:** I can explain ¹ _____, but I should not promise absolute secrecy before discussing the actual limits.
- 03 **Alex:** I would like some time without my caregiver in the room. Is that different from the record being private?
- 04 **Dr Hale:** Yes. A ² _____ and access to information afterward are separate questions.
- 05 **Alex:** My caregiver sometimes reads messages in the patient portal. Could that affect what I send?
- 06 **Dr Hale:** We need to check ³ _____ and the settings for this service rather than assume messages are invisible.
- 07 **Alex:** What about letters or insurance information that arrive at home after an appointment?
- 08 **Dr Hale:** Possible ⁴ _____ and other communications also need explanation. The contact route matters.
- 09 **Alex:** Does being sixteen mean the rules are automatically the same for every type of care?
- 10 **Dr Hale:** No. ⁵ _____ rules depend on the applicable law and the care involved.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 55.

- 11 **Alex:** I also want to understand when you might have to tell someone about a safety concern.
- 12 **Dr Hale:** We should discuss the relevant ⁶ _____ and duties before asking you for sensitive detail.
- 13 **Alex:** Would you explain what you needed to share, rather than suddenly telling me afterward?
- 14 **Dr Hale:** I will explain the process and support you as appropriate.
⁷ _____ duties, where they apply, cannot be replaced by a promise of silence.
- 15 **Alex:** I understand. Could we agree on how the service should contact me about ordinary follow-up?
- 16 **Dr Hale:** Yes. Let us confirm your ⁸ _____ and check what the service can actually provide.
- 17 **Alex:** I would rather know a limitation now than discover it after I have sent a message.
- 18 **Dr Hale:** That is why ⁹ _____ checks belong before an assurance, not after it.
- 19 **Alex:** Once those arrangements are explained, I can ask my question with a clearer understanding of the boundaries.
- 20 **Dr Hale:** Exactly. Where sharing is necessary, ¹⁰ _____ can help us explain the next step while following the actual duties.

06

ANSWERS / EXPLANATIONS: PAGE 96

1. confidentiality | 2. private consultation | 3. proxy access | 4. billing disclosure | 5. Minor consent | 6. safety exception | 7. Mandatory reporting | 8. preferred contact method | 9. record-access rule | 10. supported disclosure

A shared phone number

F Say it

A teenager says the listed phone is shared with a caregiver. The clinic has not verified a private contact route. No sensitive message has been sent.

Complete the four gaps. Use each bank entry once.

06

WORD + PHRASE BANK

shared / sent / arrangements / verified

Teenager: "That phone is ¹ _____."

Clinician: "A private route is not yet ² _____."

Teenager: "No sensitive message has been ³ _____."

Clinician: "We need to check the contact ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 96

1. shared | 2. verified | 3. sent | 4. arrangements

Rehearse with a fixed script

1. Read the corrected privacy discussion. Distinguish the room, the record, and communication channels.
2. Swap roles. Explain qualified confidentiality without stating a universal law.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

A plan must reach the people who use it

A Read the situation

Translate a clinician-approved care plan into clear responsibilities and authorized communication.

Dr Moss and school nurse Rivera discuss a student with an existing asthma diagnosis. The current clinician-approved action plan must be verified and shared through the authorized process. Rivera reports that the school holds an older version. The exercise supplies no inhaler dose, symptom threshold, or independent permission for staff to change treatment.

PEOPLE / Dr Moss: Pediatrician | Rivera: School nurse

07

SENDING IS NOT THE SAME AS IMPLEMENTATION

Check the version, recipient, authorization, and practical readiness. A document that exists but has not reached the responsible person does not complete the communication.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What does the school hold?

- A A newly verified version
- C An authorized dose change

- B A new diagnosis
- D An older action-plan version

2. What must be checked?

- A An informal verbal replacement
- C Only the student's attendance

- B The current plan and authorized sharing
- D A guessed inhaler dose

3. What is not permitted by the exercise?

- A Acknowledging receipt
- C Independent treatment changes

- B Clarifying responsibility
- D Version verification

Answers and explanations: [page 97](#).

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

asthma action plan

Individualized clinician-approved instructions for managing asthma situations.

IN USE verify the asthma action plan

controller medicine

Medicine used to manage ongoing disease control as prescribed.

IN USE explain controller medicine

reliever medicine

Medicine prescribed for relief in specified circumstances.

IN USE clarify reliever instructions

inhaler technique

Method of using the prescribed inhaler correctly.

IN USE review inhaler technique

spacer device

Holding chamber used with a compatible inhaler when prescribed.

IN USE check the spacer device

trigger exposure

Contact with a factor associated with symptoms.

IN USE describe trigger exposure

exercise-related symptom

Symptom occurring in association with physical activity.

IN USE report exercise-related symptoms

school health plan

Documented arrangements for a student's health needs at school.

IN USE update the school health plan

medication authorization

Required permission for medicine-related action in a setting.

IN USE verify medication authorization

self-carry permission

Authorization for a student to carry their own medicine.

IN USE check self-carry permission

emergency action

Required response under the actual urgent-care plan.

IN USE confirm emergency actions

plan version

Identified edition of a care document.

IN USE verify the plan version

Precision check

An older plan must not be treated as the current instruction. The exercise does not supply medicine doses or authorize staff to improvise changes.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

school contact

Named person responsible for relevant school communication.

IN USE identify the school contact

care coordination

Organization of care information and responsibilities across participants.

IN USE strengthen care coordination

activity accommodation

Appropriate adjustment supporting participation in an activity.

IN USE discuss activity accommodations

field-trip arrangement

Health-related preparation for an off-site school activity.

IN USE confirm field-trip arrangements

receipt confirmation

Acknowledgment that the intended recipient received a document.

IN USE obtain receipt confirmation

implementation readiness

Confirmation that the plan can be carried out by the appropriate people.

IN USE check implementation readiness

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

Sharing authorization, receipt, and readiness are separate checks. Use the actual emergency plan and professional scope, not the language exercise, for care.

Phrases you can actually use

C Notice the language / 1 of 2

VERIFY THE VERSION

"Which dated version does the school currently hold?"

CHECK AUTHORIZATION

"We need the appropriate permission for sharing and administration."

CONFIRM RECEIPT

"Please acknowledge receipt of the verified plan."

COVER ANOTHER SETTING

"Let us check the field-trip arrangements separately."

NAME THE MISMATCH

"That differs from the version we are reviewing."

AVOID IMPROVISATION

"Please do not substitute an unverified dose or instruction."

ASK ABOUT READINESS

"Are the responsible people and required arrangements in place?"

CLOSE THE LOOP

"We need both the current document and confirmed responsibilities."

Current versus available

A document being available does not establish that it is the current version.

Receipt versus readiness

Receiving a file does not prove staff and equipment are prepared to implement it.

Separate arrangements

Another setting may require its own practical preparation rather than an assumed copy of the classroom process.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which question addresses version control?

- A** Which dated plan does the school hold?
- B** Which color is the folder?
- C** Can we use any old dose?
- D** Does receipt prove readiness?

2. What is not equivalent to implementation?

- A** Verifying the current plan
- B** Checking actual readiness
- C** Sending the document
- D** Confirming authorized responsibilities

3. Which instruction is unsupported?

- A** Confirm receipt.
- B** Increase the inhaler dose from this exercise.
- C** Verify the plan.
- D** Check authorization.

4. Why check a field trip separately?

- A** Permission never matters outside school.
- B** An old plan automatically becomes current.
- C** The practical setting and arrangements may differ.
- D** The diagnosis disappears off-site.

[Answers and explanations: page 97.](#)

A plan must reach the people who use it

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

Self-carry permission / medication authorization / receipt confirmation / care coordination / asthma action plan / plan version / emergency action / activity accommodation / Implementation readiness / field-trip arrangement

07

- 01 **Rivera:** The school has an asthma plan, but its date is older than the version the family mentioned this morning.
- 02 **Dr Moss:** Let us verify the ¹ _____ before anyone assumes the instructions are unchanged.
- 03 **Rivera:** I would like the current document through the authorized route, not a partial photograph from another parent.
- 04 **Dr Moss:** Agreed. We need the verified ² _____ and appropriate permission for sharing it.
- 05 **Rivera:** We also need to know which staff are authorized to assist and what permissions apply.
- 06 **Dr Moss:** Check the actual ³ _____ and professional responsibilities rather than infer them from the document's presence.
- 07 **Rivera:** The family asked whether the student can carry the inhaler independently at school.
- 08 **Dr Moss:** ⁴ _____ depends on the applicable arrangements. This discussion does not grant it automatically.
- 09 **Rivera:** Once the verified plan arrives, I will acknowledge that the intended school contact has received it.
- 10 **Dr Moss:** That ⁵ _____ is useful, but we should separately confirm practical readiness.

The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 64.

- 11 **Rivera:** You mean the people, equipment, and procedures needed to carry out the actual plan?
- 12 **Dr Moss:** Yes. ⁶ _____ is more than keeping a file in the school record.
- 13 **Rivera:** There is also a field trip next week, which has a different staffing arrangement.
- 14 **Dr Moss:** Then check the ⁷ _____ through the appropriate process rather than assume the classroom plan transfers unchanged.
- 15 **Rivera:** I will use the child's current signed plan and clarify anything that does not match the instructions we hold.
- 16 **Dr Moss:** Correct. The actual ⁸ _____ comes from the verified plan and real clinical process.
- 17 **Rivera:** The student also wants to participate in activities without being treated as a problem.
- 18 **Dr Moss:** Appropriate ⁹ _____ can be discussed without assuming exclusion or promising unrestricted participation.
- 19 **Rivera:** I will confirm the version, authorization, receipt, and any outstanding practical issue through the designated contact.
- 20 **Dr Moss:** That is effective ¹⁰ _____: a usable current plan and clear responsibilities across the family, school, and clinical team.

07

ANSWERS / EXPLANATIONS: PAGE 98

1. plan version | 2. asthma action plan | 3. medication authorization | 4. Self-carry permission | 5. receipt confirmation | 6. Implementation readiness | 7. field-trip arrangement | 8. emergency action | 9. activity accommodation | 10. care coordination

The new file arrived but training is pending

F Say it

The school confirms receipt of the verified current plan. The responsible staff still need the required preparation. Implementation readiness is not yet confirmed.

Complete the four gaps. Use each bank entry once.

07

WORD + PHRASE BANK

arrangements / pending / received / readiness

Nurse: "The current plan has been ¹ _____."

Clinician: "Staff preparation remains ² _____."

Nurse: "Receipt does not prove ³ _____."

Clinician: "Please confirm the remaining ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 98

1. received | 2. pending | 3. readiness | 4. arrangements

Rehearse with a fixed script

1. Read the corrected school exchange, separating version, permission, receipt, and readiness.
2. Swap roles. Confirm the communication process without adding medicine instructions.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Different from the infant's usual behavior

A Read the situation

Lead with a concerning change and preserve the caregiver's account during an active urgent response.

Caregiver Mei reports that infant Arlo is much less responsive than usual and is feeding poorly. The actual emergency response is already active; this language exercise must not delay it. Dr Reed receives the account and transfers the relevant information to the clinical team. No diagnosis, measured observations, or safe waiting interval is supplied.

PEOPLE / Mei: Caregiver | Dr Reed: Pediatrician

08

TAKE A CHANGE FROM USUAL SERIOUSLY

A caregiver can identify an important change without having clinical vocabulary. Ask for concrete observations while the real response proceeds, and do not replace uncertainty with reassurance.

Briefing checks

Circle one answer in each item. Use the briefing.

1. What change does Mei report?

- A** A safe waiting period
- C** A confirmed diagnosis

- B** Much less responsiveness and poor feeding
- D** Normal measured observations

2. What is already active?

- A** A routine appointment next month
- C** A language-only review

- B** A completed discharge
- D** The emergency response

3. What must remain unknown here?

- A** Mei's report
- C** The need to preserve the account

- B** Arlo's name
- D** The diagnosis and measured observations

Answers and explanations: page 99.

The vocabulary that does the work

B Find the words / 1 of 2

Read the definitions and say the common word combinations aloud.

responsiveness

Degree of response to people or stimulation in context.

IN USE describe reduced responsiveness

poor feeding

Feeding less effectively or less than usual as reported or assessed.

IN USE report poor feeding

usual pattern

Person's typical behavior used as a comparison.

IN USE establish the usual pattern

caregiver concern

Worry expressed by the person caring for the child.

IN USE acknowledge caregiver concern

acute change

New or recent difference from the prior state.

IN USE communicate an acute change

observed behavior

Action or response personally witnessed by the reporter.

IN USE describe observed behavior

last known usual state

Most recent time the person was observed in their usual state.

IN USE clarify the last known usual state

measured observation

Value obtained through an actual assessment or measurement.

IN USE relay measured observations

work of breathing

Effort involved in breathing as clinically assessed.

IN USE assess work of breathing

hydration assessment

Clinical evaluation of fluid status using relevant information.

IN USE perform a hydration assessment

urine output

Amount or pattern of urine production over time.

IN USE clarify urine output

febrile illness

Illness involving fever in the relevant clinical context.

IN USE assess a febrile illness

08

Precision check

Less responsive than usual is the caregiver's important report, not a measured consciousness score. Preserve the account while the actual emergency assessment proceeds.

The vocabulary that does the work

B Find the words / 2 of 2

Read the definitions and say the common word combinations aloud.

clinical deterioration

Worsening of a person's clinical condition.

IN USE communicate suspected clinical deterioration

urgent escalation

Prompt transfer of a concern through the appropriate clinical route.

IN USE initiate urgent escalation

emergency response

Active process for addressing an urgent clinical situation.

IN USE confirm the emergency response

receiving clinician

Professional accepting the relevant care information and responsibility.

IN USE identify the receiving clinician

handoff priority

Most important information to lead a transfer of care.

IN USE state the handoff priority

read-back confirmation

Repetition of key information to verify accurate receipt.

IN USE obtain read-back confirmation

qualified interpreter

Trained language professional supporting accurate communication between speakers.

IN USE request a qualified interpreter

teach-back

Checking an explanation by inviting the listener to restate its meaning.

IN USE use teach-back without blame

patient preference

A person's stated wishes about care or communication.

IN USE elicit the patient preference

clinical handoff

Transfer of relevant care information and responsibility between professionals.

IN USE confirm the clinical handoff

documented plan

Agreed next steps recorded for the people involved.

IN USE review the documented plan

source attribution

Identification of who reported, observed, or established a fact.

IN USE preserve source attribution

Precision check

The absence of supplied measurements does not make the situation reassuring. Do not use the exercise to choose a diagnosis or a waiting interval.

Phrases you can actually use

C Notice the language / 1 of 2

LEAD WITH THE CHANGE

"You are reporting a clear change from Arlo's usual behavior."

ASK CONCRETELY

"What response did you see when you spoke to Arlo?"

AVOID FALSE REASSURANCE

"We do not have a diagnosis or verified measurements to explain this yet."

IDENTIFY THE RECEIVER

"I will confirm which clinician is receiving the information."

CONFIRM ACTION

"The emergency response is already active."

PRESERVE THE SOURCE

"I will pass on exactly what you observed."

KEEP URGENCY SEPARATE

"Do not wait to complete this conversation before following the emergency process."

CHECK THE MESSAGE

"Please repeat the key change so we can confirm accurate receipt."

08

Usual versus normal

Usual compares with this infant's baseline; normal would imply a broader clinical assessment.

Reports versus measured

A caregiver report and a measured observation have different sources.

Already active

Keeps the actual emergency process ahead of completing the language task.

Phrases you can actually use

C Notice the language / 2 of 2

INVITE CORRECTION

"Please stop me if I have misunderstood."

EXPLAIN A TERM

"In everyday language, that means the following."

SEPARATE UNCERTAINTY

"That remains uncertain; it is not a confirmed finding."

CHECK YOUR EXPLANATION

"Let me explain that another way."

MAKE SPACE

"We can pause before moving to the next point."

NAME RESPONSIBILITY

"Let us confirm who will follow up."

KEEP A RECORD ACCURATE

"I will distinguish what was reported from what was observed."

CONFIRM AN AGREEMENT

"We have agreed on the next step, not on an outcome."

Reported versus established

Attribute a person's account with reports or describes. Use confirms only when the relevant evidence supports confirmation.

Can versus will

Can describes an available possibility or ability; will makes a commitment. Do not turn a possible result into a promise.

Checking a message

A check-back confirms the meaning received. Correct the explanation calmly when the listener has understood something different.

Choose the precise message

D Check your understanding

Circle one best answer for each item. Use the stated purpose and case facts, not the option that merely sounds most polite.

1. Which opening conveys the priority?

- A** Everything is normal because no numbers are shown.
- B** A definite diagnosis has been established.
- C** Arlo is much less responsive than usual and feeding poorly.
- D** There may be some paperwork.

2. Which statement is unsupported?

- A** The receiving team needs the account.
- B** Arlo's measured observations are normal.
- C** Mei reports a change.
- D** Emergency response is active.

3. What should not delay care?

- A** Completing the language exercise
- B** The actual emergency response
- C** Necessary clinical assessment
- D** Appropriate urgent action

4. What does read-back accomplish?

- A** Replaces clinical examination
- B** Guarantees recovery
- C** Checks accurate receipt of key information
- D** Establishes a diagnosis automatically

Answers and explanations: page 99.

Different from the infant's usual behavior

E Conversations / turns 1-10

Complete the numbered gaps. Use each bank entry once, without changing its form. Cover the answers at the end until you finish.

WORD + PHRASE BANK

emergency response / responsiveness / receiving clinician / poor feeding / measured observation / observed behavior / caregiver concern / read-back confirmation / handoff priority / last known usual state

08

- 01 **Mei:** Arlo is much less responsive than usual and has been feeding poorly. This is very different from the usual pattern.
- 02 **Dr Reed:** I hear the change. The ¹ _____ is already active, and the clinical team is receiving the concern.
- 03 **Mei:** I do not know the medical word. Arlo does not respond to my voice in the usual way.
- 04 **Dr Reed:** That description of ² _____ is useful. Tell us what you observed rather than trying to choose a diagnosis.
- 05 **Mei:** I can describe what I saw, but I have not measured anything or kept a formal chart.
- 06 **Dr Reed:** We will distinguish your ³ _____ report from any measurements the team obtains during assessment.
- 07 **Mei:** The feeding is also different. I do not want that detail to disappear when someone else takes over.
- 08 **Dr Reed:** I will include ⁴ _____ in the handoff alongside the reduced responsiveness.
- 09 **Mei:** Should I try to remember the exact time everything changed before the team continues?
- 10 **Dr Reed:** Clarify the ⁵ _____ as accurately as you can, but do not delay the active response to produce a perfect timeline.



The conversation, continued

E Conversations / turns 11-20

Continue with the word bank on page 73.

- 11 **Mei:** I am certain this is different from yesterday, even if I cannot give an exact minute.
- 12 **Dr Reed:** That ⁶ _____ matters. We will preserve the uncertainty about timing instead of replacing it with a guessed value.
- 13 **Mei:** Will the next clinician hear my words or only a short label that loses the change?
- 14 **Dr Reed:** The ⁷ _____ is the change you described, with its source and what is still unknown.
- 15 **Mei:** I do not want someone to assume that missing numbers mean all the observations are normal.
- 16 **Dr Reed:** They do not. A ⁸ _____ requires an actual assessment; missing information is not reassurance.
- 17 **Mei:** Who will be responsible for receiving the update as the assessment continues?
- 18 **Dr Reed:** We will identify the ⁹ _____ through the active process and confirm the transfer.
- 19 **Mei:** I can repeat the main change if that helps make sure it was heard correctly.
- 20 **Dr Reed:** A ¹⁰ _____ helps: reduced responsiveness compared with usual, poor feeding, and the emergency process continuing now.

08

ANSWERS / EXPLANATIONS: PAGE 100

1. emergency response | 2. responsiveness | 3. observed behavior | 4. poor feeding | 5. last known usual state | 6. caregiver concern | 7. handoff priority | 8. measured observation | 9. receiving clinician | 10. read-back confirmation

The timeline is corrected during care

F Say it

During an active emergency response, the caregiver corrects the last observed usual state from 08:00 to 07:30. The receiving clinician acknowledges the correction. No diagnosis is supplied.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

08:00 / active / unestablished / 07:30

08

Caregiver: "The corrected time is ¹ _____."

Clinician: "The earlier report of ² _____ is superseded."

Caregiver: "The emergency response remains ³ _____."

Clinician: "The diagnosis remains ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 100

1. 07:30 | 2. 08:00 | 3. active | 4. unestablished

Rehearse with a fixed script

1. Read the corrected handoff, leading with the two reported changes and the active response.
2. Swap roles. Preserve uncertain timing without inventing measurements or a safe waiting interval.
3. Complete the four-line follow-up with the supplied words. Check every explanation, then read the corrected exchange twice, changing roles.

WORKING ALONE

Read both roles, pausing for two seconds when the speaker changes. Repeat with the bank covered, then check any missed expressions.

Preparing for an adult-care transition

E More workplace conversations / 1-10

Build a verified transition plan without treating a referral as completed transfer.

Seventeen-year-old Kai and pediatrician Dr Reed prepare for a future move to adult services. The receiving service has not confirmed an appointment. Kai wants to understand the condition summary, medicine list, contact routes, and changing communication responsibilities. No universal age rule or legal conclusion is supplied.

PEOPLE / Kai: Adolescent patient | Dr Reed: Pediatrician

Use each bank entry once, without changing its form. Answers follow turn 20.

09

WORD + PHRASE BANK

communication arrangements / condition summary / receiving service / transition plan / readiness discussion / confirmed transfer

- 01 **Kai:** I heard that I will move to an adult clinic, but I do not know what I need to understand before that happens.
- 02 **Dr Reed:** Let us prepare a ¹ _____ with clear steps rather than assume that sending a referral completes the change.
- 03 **Kai:** I want to explain my health history myself, but I do not remember every technical term.
- 04 **Dr Reed:** A verified ² _____ can help you describe the important facts without memorizing the whole record.
- 05 **Kai:** Could we review which medicines are current instead of copying an old list into the new form?
- 06 **Dr Reed:** Yes. We should verify the actual list and instructions with the responsible team before presenting them as current.
- 07 **Kai:** I also want to know whom to contact if the adult clinic has not offered an appointment yet.
- 08 **Dr Reed:** The ³ _____ has not confirmed an appointment, so we need a named route for checking progress.
- 09 **Kai:** Does that mean the pediatric team stops answering questions as soon as the referral is sent?
- 10 **Dr Reed:** We need explicit coverage and responsibility arrangements. They should not be assumed from the date a letter was sent.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 76.

- 11 **Kai:** Some messages currently go to my caregiver. Will that automatically change on a particular birthday?
- 12 **Dr Reed:** The actual ⁴ _____ and applicable rules need review; we should not invent a universal age rule.
- 13 **Kai:** I would like help learning to ask questions, but I do not want my caregiver excluded without discussion.
- 14 **Dr Reed:** We can discuss the support you want and the appropriate permissions while keeping you involved in your care.
- 15 **Kai:** How do we know whether I am ready to manage a particular task independently?
- 16 **Dr Reed:** A ⁵ _____ looks at understanding and practical support needs, not just a birthday or a form.
- 17 **Kai:** Could we check the emergency and routine contact routes separately? I sometimes confuse them.
- 18 **Dr Reed:** Yes. The actual urgent-care instructions remain distinct from routine appointment and prescription questions.
- 19 **Kai:** Then we can confirm each handoff step instead of assuming the new clinic already knows everything.
- 20 **Dr Reed:** A ⁶ _____ requires accepted responsibility and clear arrangements, not simply a sent letter.

09

ANSWERS / EXPLANATIONS: PAGE 101

1. transition plan | 2. condition summary | 3. receiving service | 4. communication arrangements | 5. readiness discussion | 6. confirmed transfer

A referral is received but no visit is booked

F Say it in a new situation

The adult service confirms receipt of Kai's referral, but no appointment is booked. The teams clarify interim responsibility under their actual process.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

letter / booked / received / clarified

Kai: "The referral has been ¹ _____."

Pediatrician: "A visit is not yet ² _____."

Kai: "Interim responsibility needs to be ³ _____."

Pediatrician: "The transfer is not simply a sent ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 101

1. received | 2. booked | 3. clarified | 4. letter

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

A school report with limited sharing permission

E More workplace conversations / 1-10

Clarify the purpose, audience, and authorized scope of a school-related medical report.

Caregiver Mina asks Dr Cole for information supporting a school accommodation. The school asks for the entire medical record, but the family has authorized discussion of the relevant functional needs only. The clinician will verify applicable requirements and permissions. No legal entitlement or educational placement decision is supplied.

PEOPLE / Mina: Caregiver | Dr Cole: Pediatrician

Use each bank entry once, without changing its form. Answers follow turn 20.

10

WORD + PHRASE BANK

unsupported statement / purpose-limited disclosure / functional need / information-sharing permission / report scope / designated contact

- 01 **Mina:** The school wants information for an accommodation, but its form asks for the entire medical record. That feels broader than we discussed.
- 02 **Dr Cole:** Let us clarify the purpose, actual requirements, and authorization before sending information beyond the relevant needs.
- 03 **Mina:** We wanted them to understand the difficulty with a particular school activity, not every past diagnosis.
- 04 **Dr Cole:** The ¹ _____ should be described using supported facts and the appropriate clinical assessment.
- 05 **Mina:** Does agreeing to a school conversation automatically authorize every teacher to see all the records?
- 06 **Dr Cole:** No. We need to check the ² _____ and applicable rules for the actual information and recipients.
- 07 **Mina:** I would like the report to explain the need without guaranteeing an educational decision you do not control.
- 08 **Dr Cole:** That is an important distinction. A clinical report can inform a process without deciding the school's response or legal obligations.
- 09 **Mina:** Could the school ask for clarification if the wording is too general to be useful?
- 10 **Dr Cole:** Yes, through the appropriate ³ _____ and authorized route, rather than distributing sensitive details to everyone.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 79.

- 11 **Mina:** We should also involve my child appropriately, because the proposed support affects the school day.
- 12 **Dr Cole:** Agreed. We can explain the discussion in an age-appropriate way and hear the child's perspective.
- 13 **Mina:** If a requested statement has not been assessed, should we include it because the form has a box for it?
- 14 **Dr Cole:** No. An ⁴ _____ should not become part of the report simply to complete a form.
- 15 **Mina:** Then the report can say what is established and what remains outside the current assessment.
- 16 **Dr Cole:** Exactly. The ⁵ _____ should remain clear, with relevant limitations stated.
- 17 **Mina:** I would like to know which version was sent and to which person, so we can follow up consistently.
- 18 **Dr Cole:** We can use the approved documentation process to record the recipient, purpose, and relevant version.
- 19 **Mina:** That gives the school useful information without treating a limited request as permission for everything.
- 20 **Dr Cole:** A ⁶ _____ supports that aim while following the actual rules and clinical responsibilities.

10

ANSWERS / EXPLANATIONS: PAGE 102

1. functional need | 2. information-sharing permission | 3. designated contact | 4. unsupported statement | 5. report scope | 6. purpose-limited disclosure

A new recipient requests the same report

F Say it in a new situation

A staff member not named in the original sharing permission requests the report. The clinician checks authorization before sending it. The report has not been sent to that person.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

authorization / unlimited / named / sent

10

Caregiver: "That recipient was not ¹ _____."

Clinician: "We need to check ² _____."

Caregiver: "The report has not been ³ _____ to them."

Clinician: "The original permission was not ⁴ _____."

ANSWERS / EXPLANATIONS: PAGE 102

1. named | 2. authorization | 3. sent | 4. unlimited

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

Reconciling two feeding accounts

E More workplace conversations / 1-10

Clarify amounts, timing, and sources without treating different caregiver accounts as deception.

Pediatrician Dr Noor discusses an infant's feeding history with caregiver Alex. Another caregiver describes a different feeding pattern. The accounts cover different times of day and may use different units. No feeding prescription, growth diagnosis, or safe waiting interval is supplied.

PEOPLE / Alex: Caregiver | Dr Noor: Pediatrician

Use each bank entry once, without changing its form. Answers follow turn 20.

WORD + PHRASE BANK

estimated amount / measurement unit / source-specific summary / observation window / history
reconciliation / Offered volume

11

- 01 **Alex:** My partner's feeding account sounds different from mine. I am worried that the difference makes one of us look unreliable.
- 02 **Dr Noor:** Let us clarify the times, units, and what each person observed before deciding that the accounts conflict.
- 03 **Alex:** I usually describe the daytime feeds, while my partner is remembering the night.
- 04 **Dr Noor:** That ¹ _____ matters. Different periods may produce different but accurate descriptions.
- 05 **Alex:** I also said ounces once and milliliters another time, which may have made the conversation confusing.
- 06 **Dr Noor:** We should identify the ² _____ explicitly rather than compare numbers that represent different quantities.
- 07 **Alex:** Sometimes I describe how much was offered, but my partner reports what seemed to be taken.
- 08 **Dr Noor:** ³ _____ and the amount actually taken are not automatically the same.
- 09 **Alex:** That distinction is useful. We should not present every number as a precise measurement if it was only an estimate.
- 10 **Dr Noor:** Correct. Mark an ⁴ _____ as an estimate, and explain how it was obtained when relevant.

The conversation, continued

E More workplace conversations / 11-20

Continue with the word bank on page 82.

- 11 **Alex:** Would a note about a difficult feed matter even if it does not fit neatly into a volume total?
- 12 **Dr Noor:** Describe the actual difficulty and associated observations. A history is more than adding numbers together.
- 13 **Alex:** I do not want this discussion to become a new feeding plan before you have assessed the actual situation.
- 14 **Dr Noor:** It should not. We are clarifying the account; any clinical recommendation needs the appropriate assessment and current information.
- 15 **Alex:** Could we summarize daytime and nighttime separately and then check the account with both caregivers?
- 16 **Dr Noor:** A ⁵ _____ helps preserve who reported each detail.
- 17 **Alex:** If we notice a concerning change, we should follow the actual clinical instructions rather than wait to perfect a diary.
- 18 **Dr Noor:** Yes. Necessary assessment must not be delayed to complete a language task or produce a flawless record.
- 19 **Alex:** Then the different accounts can be clarified without accusing either of us or inventing an average.
- 20 **Dr Noor:** That is ⁶ _____: check the sources and meanings before drawing a clinical conclusion.

11

ANSWERS / EXPLANATIONS: PAGE 103

1. observation window | 2. measurement unit | 3. Offered volume | 4. estimated amount | 5. source-specific summary | 6. history reconciliation

Offered and taken were confused

F Say it in a new situation

One caregiver reports the amount offered, while another reports the amount taken. The units are the same, but the quantities describe different things. No intake total is calculated.

Complete the four gaps. Use each bank entry once.

WORD + PHRASE BANK

same / calculated / taken / offered

Caregiver: "My figure describes what was ¹ _____."

Clinician: "The other describes what was ² _____."

Caregiver: "The units are the ³ _____."

Clinician: "No combined total has been ⁴ _____."

11

ANSWERS / EXPLANATIONS: PAGE 103

1. offered | 2. taken | 3. same | 4. calculated

Read, switch, repeat

1. Check the four answers and read the complete exchange aloud.
2. Switch roles. Read it again with the word bank covered.
3. Return to the extended conversation. Read turns 1-10, then switch roles for turns 11-20. Use the corrected expressions.

SOLO PRACTICE

Read both roles. Pause briefly at each change of speaker. Repeat any line that needed an answer check.

01 Briefing and language checks

Hearing the child and caregiver

A Briefing checks / page 4

1. When does Leo report discomfort?

C. Before school

Leo describes discomfort before school in the supplied brief.

2. What does the caregiver observe?

C. Discomfort after dinner

The caregiver reports seeing episodes after dinner, not every possible episode.

3. What is not established?

A. The cause

The case does not supply a cause, examination finding, or diagnosis.

D Language checks / page 9

1. Which summary preserves the sources?

C. Leo reports morning episodes; his caregiver notices evening episodes.

The summary keeps both accounts visible without assigning cause or credibility.

2. Which question clarifies duration?

A. How long does one episode last?

Duration concerns the length of one occurrence rather than its frequency or assumed cause.

3. Which conclusion exceeds the facts?

B. Leo has a school-avoidance diagnosis.

No diagnosis or motive is established by the supplied symptom timing.

4. What is a summary check for?

D. Correcting the clinician's understanding

The check verifies whether the clinician has understood the account accurately.

01 Conversation answers

Hearing the child and caregiver

E Dialogue gaps

Pages 10-11

1. child account. Turn 3. The child account is Leo's own description and should be heard directly rather than replaced by an adult's interpretation.

2. caregiver account. Turn 5. The caregiver account records what the caregiver reports and may concern different episodes or settings from the child's description.

3. symptom frequency. Turn 7. Symptom frequency means how often the discomfort occurs and differs from the length of any one episode.

4. recalled event. Turn 9. A recalled event is described from memory and should not be presented as a complete contemporaneous record.

5. functional impact. Turn 11. Functional impact concerns how the symptom affects everyday activities rather than assuming its effect from the label alone.

6. contextual difference. Turn 13. A contextual difference concerns the circumstances of separate accounts and does not automatically reveal a motive or diagnosis.

7. developmental language. Turn 15. Developmental language adapts the discussion to the child's understanding while preserving the meaning of the child's account.

8. Direct observation. Turn 17. Direct observation identifies what a speaker personally witnessed and should be distinguished from symptoms another person experiences privately.

9. episode duration. Turn 19. Episode duration describes the length of one occurrence and must not be confused with onset or how often symptoms happen.

10. summary check. Turn 20. A summary check invites correction of the clinician's understanding rather than requiring the child to agree with an inaccurate account.

F Transfer conversation

Page 12

1. lunchtime

Teacher: "I noticed the discomfort at lunchtime."

The teacher's observation is explicitly located at lunchtime.

2. account

Clinician: "That is the teacher's account."

The information comes from the teacher rather than a completed clinical assessment.

3. discussed

Leo: "We have not yet discussed that episode."

The scenario leaves Leo's account of this particular episode uncollected.

4. unknown

Clinician: "The cause remains unknown."

A new observation does not establish the cause of the discomfort.

02 Briefing and language checks

Explaining a growth trajectory

A Briefing checks / page 13

1. What does Ana mistake the percentile for?

A. A score out of 100

Ana interprets the percentile as a school-style score rather than a comparison.

2. What still needs review?

B. The trajectory and clinical context

The case requires the pattern, measurement quality, and context to be reviewed.

3. What is not diagnosed?

D. A growth disorder

The brief supplies a terminology misunderstanding but no growth-disorder diagnosis.

D Language checks / page 18

1. Which explanation is accurate?

B. A percentile locates a measurement within a reference distribution.

A percentile is a comparison position, not a grade, completion percentage, or universal target.

2. What does growth velocity describe?

C. Rate of growth over time

Growth velocity concerns change over time rather than a single plotted position.

3. Which check improves interpretation?

D. Verify measurement technique and prior points.

Reliable measurements and their sequence support the actual clinical interpretation.

4. Which conclusion is unsupported?

D. This child has a diagnosed growth disorder.

No growth-disorder diagnosis has been supplied in the case.

02 Conversation answers

Explaining a growth trajectory

E Dialogue gaps

Pages 19-20

- 1. percentile.** Turn 2. A percentile is a reference comparison, not a grade for the child.
- 2. reference population.** Turn 4. The reference population supplies the comparison, not a universal growth target.
- 3. growth trajectory.** Turn 6. Growth trajectory describes measurements across time rather than one isolated point.
- 4. serial measurements.** Turn 8. Serial measurements need timing and comparability checks before interpreting a pattern.
- 5. plotting error.** Turn 10. A plotting error changes the chart entry, not necessarily the child's growth.
- 6. Growth velocity.** Turn 12. Growth velocity is the rate of change over time, whereas a percentile describes a measurement's position relative to a reference.
- 7. longitudinal assessment.** Turn 14. A longitudinal assessment uses information across time rather than treating one shared percentile as a complete account of growth.
- 8. measurement technique.** Turn 16. Measurement technique is the method used to obtain the value and affects how reliably it can be interpreted.
- 9. nutritional history.** Turn 18. A nutritional history describes intake and related circumstances and contributes to assessment without being a diagnosis by itself.
- 10. centile crossing.** Turn 20. Centile crossing describes movement across reference lines and requires contextual review rather than an automatic diagnosis from the chart alone.

F Transfer conversation

Page 21

1. incorrect

Caregiver: "The copied height may be incorrect."

The scenario identifies a possible error in the copied height.

2. measurement

Clinician: "We are checking the original measurement."

The original measurement is the source being verified.

3. interpreted

Caregiver: "The trajectory is not yet interpreted."

Interpretation waits for the relevant data verification in this case.

4. disorder

Clinician: "An entry error is not itself a growth disorder."

A data-entry error does not establish a clinical growth disorder.

03 Briefing and language checks

Discussing developmental screening

A Briefing checks / page 22

1. What raises the concern?

A. A developmental questionnaire

The questionnaire flags a communication concern without establishing a diagnosis.

2. What does Ben fear?

D. That a lifelong diagnosis is already settled

Ben interprets the screening concern as a completed diagnosis.

3. What is not supplied?

D. A specific diagnosis or appointment date

The case does not provide a diagnostic conclusion or confirmed referral appointment.

D Language checks / page 27

1. Which statement preserves the distinction?

B. Screening flags a concern; evaluation investigates it.

Screening and diagnostic evaluation have different roles and should not be treated as equivalent.

2. Which question clarifies regression?

A. Was the skill previously present and then lost?

Regression concerns loss of a previously acquired ability rather than simply a skill not yet acquired.

3. How should multilingual exposure be handled?

A. Describe it accurately within the assessment.

The child's language experience belongs in the assessment without becoming an automatic causal explanation.

4. Which referral claim is unsupported?

A. An appointment is already booked.

The case gives no confirmed appointment date or booking.

03 Conversation answers

Discussing developmental screening

E Dialogue gaps

Pages 28-29

1. Developmental screening. Turn 2.

Developmental screening flags concerns for assessment; it does not establish diagnosis.

2. diagnostic evaluation. Turn 4. A diagnostic evaluation examines additional information rather than treating screening as definitive.

3. Receptive language. Turn 6. Receptive language concerns understanding, whereas expressive language concerns communicating ideas.

4. multilingual exposure. Turn 8. Multilingual exposure belongs in the history but does not explain every concern.

5. social communication. Turn 10. Social communication includes interaction and is broader than counting spoken words.

6. Skill regression. Turn 12. Skill regression means loss of a previously acquired ability and differs from a skill that has not yet emerged.

7. hearing assessment. Turn 14. A hearing assessment evaluates hearing and may contribute to investigating communication concerns rather than being replaced by an informal impression.

8. early intervention. Turn 16. Early intervention addresses developmental needs early in childhood, with actual eligibility and services determined through the relevant local process.

9. referral status. Turn 18. Referral status identifies the current processing stage and should distinguish a sent referral from receipt or a booked appointment.

10. developmental surveillance. Turn 20. Developmental surveillance follows development across encounters and complements, rather than replaces, screening and appropriate diagnostic assessment.

F Transfer conversation

Page 30

1. receipt

Caregiver: "The service has confirmed receipt."

The supplied update confirms that the referral reached the service.

2. booked

Clinician: "An appointment is not yet booked."

Receipt does not establish that an appointment has been scheduled.

3. contact

Caregiver: "We have a progress-check contact."

The scenario provides a contact for checking the referral's progress.

4. pending

Clinician: "The diagnostic conclusion remains pending."

No completed diagnostic assessment or conclusion is supplied in this update.

04 Briefing and language checks

Responding to vaccine questions

04

A Briefing checks / page 31

1. What worries Dana?

A. Fever after a previous vaccination

Dana asks about fever after a previous vaccination and whether it will recur.

2. What needs review?

A. The event history and actual recommendation

The clinician reviews the individual account and current recommendation rather than assuming an answer.

3. What is not supplied?

D. A product-specific risk estimate

No product-specific estimate is available in this language case.

D Language checks / page 36

1. Which wording preserves causation uncertainty?

A. Fever occurred after vaccination; its cause needs review.

The wording acknowledges the event while keeping the causal question open for review.

2. Which claim is unsupported?

B. No reaction can ever occur.

A universal zero-risk promise cannot be justified by this scenario.

3. What is an adverse event?

C. An unwanted occurrence not necessarily caused by the intervention

An event after an intervention and a causally attributed reaction are not automatically identical.

4. Which question is most useful first?

D. What happened and when?

Specific event and timing details support assessment more effectively than labeling the caregiver.

04 Conversation answers

Responding to vaccine questions

E Dialogue gaps

Pages 37-38

1. vaccination history. Turn 2. Vaccination history records prior administrations and supports review of the reported event.

2. adverse event. Turn 4. An adverse event occurs after intervention without necessarily being caused by it.

3. temporal association. Turn 6. Temporal association establishes sequence, not proof that one event caused another.

4. causal relationship. Turn 8. A causal relationship needs assessment beyond merely identifying which event happened first.

5. contraindication. Turn 10. A contraindication requires verification against the actual history and applicable guidance.

6. vaccine recommendation. Turn 12. A vaccine recommendation is current advice for a product and context and should be explained with its relevant benefits, limitations, and risks.

7. absolute risk. Turn 14. Absolute risk gives the probability or frequency for a defined outcome, group, and period, rather than a context-free percentage.

8. administration record. Turn 16. An administration record documents what was actually given and should be checked before missing entries are treated as completed or omitted doses.

9. catch-up schedule. Turn 18. A catch-up schedule addresses missed doses under current guidance and requires the actual verified history rather than an improvised sequence.

10. informed choice. Turn 20. An informed choice follows relevant information and questions being addressed rather than pressure to agree before the concern is understood.

F Transfer conversation

Page 39

1. unclear

Caregiver: "This entry is unclear."

The outside record contains an entry whose meaning is unresolved.

2. clarification

Clinician: "We have requested clarification."

The clinic asks the originating source to clarify the entry.

3. confirmed

Caregiver: "Administration is not yet confirmed."

An unclear entry is not proof that the vaccine was administered.

4. record

Clinician: "We will use the verified record."

The next decision uses the clarified actual record rather than an assumption.

05 Briefing and language checks

Checking liquid-medicine instructions

A Briefing checks / page 40

1. Which units does Omar confuse?

D. mg and mL

Omar treats milligrams and milliliters as interchangeable even though they measure different quantities.

2. What must be checked?

B. The actual bottle, prescription, and measuring tool

The case requires the actual verified product and instructions rather than a substitute.

3. What is deliberately not supplied?

D. A numerical dose

No dose or administration instruction is supplied for the learner to use clinically.

D Language checks / page 45

1. Which statement is correct?

A. mg measures mass; mL measures volume.

The units describe different quantities, linked by the actual product concentration.

2. What must not be guessed?

A. The concentration

A guessed concentration can make an amount-to-volume interpretation unsafe and is not supplied here.

3. Which tool is inappropriate for precise measurement?

D. An ordinary kitchen spoon

Household spoons vary and are not reliable substitutes for the appropriate medicine-measuring device.

4. What is the purpose of demonstration?

D. Check understanding of verified instructions

A demonstration checks how the explanation was understood using the real verified instructions.

05 Conversation answers

Checking liquid-medicine instructions

E Dialogue gaps

Pages 46-47

- 1. milligram.** Turn 2. A milligram measures drug mass; a milliliter measures liquid volume.
- 2. concentration.** Turn 4. Concentration links drug amount and volume and must be verified.
- 3. prescription label.** Turn 6. The current prescription label cannot be replaced by another bottle's remembered instructions.
- 4. oral syringe.** Turn 8. An oral syringe must suit the actual verified liquid-medicine instruction.
- 5. calibration mark.** Turn 10. Calibration marks indicate measured quantities and must be readable and appropriate to the intended measurement.
- 6. label discrepancy.** Turn 12. A label discrepancy is a conflict between instructions that needs clarification with the responsible prescribing or dispensing team.
- 7. dispensing pharmacy.** Turn 14. The dispensing pharmacy supplied the product and is an appropriate contact for verifying its label, concentration, and measuring device.
- 8. active ingredient.** Turn 16. The active ingredient is the pharmacologically active substance and should be checked across products rather than relying only on brand names.
- 9. caregiver demonstration.** Turn 18. A caregiver demonstration shows how verified instructions are understood and allows the explanation or equipment to be corrected before use.
- 10. dose volume.** Turn 20. Dose volume is the liquid quantity corresponding to a verified dose and concentration; it is not supplied or calculated in this exercise.

F Transfer conversation

Page 48

1. different

Caregiver: "The brand names are different."

The products have different brand names in the supplied scenario.

2. unchecked

Clinician: "The active ingredients remain unchecked."

Different brands do not establish whether their active ingredients differ.

3. duplication

Caregiver: "We should not assume there is no duplication."

The ingredients must be reviewed before excluding duplication.

4. review

Clinician: "The responsible team will review them."

The next step is the actual product review, not a guessed administration plan.

05

06 Briefing and language checks

Explaining adolescent confidentiality

A Briefing checks / page 49

1. What does Alex ask?

B. Whether everything will remain private

Alex asks about the limits of privacy before sensitive discussion.

2. What does Dr Hale check?

A. Actual access arrangements and applicable rules

Privacy depends on the real service, records, and applicable requirements.

3. What is not supplied?

D. A jurisdiction-specific legal conclusion

The case does not determine the legal rules for a particular jurisdiction.

D Language checks / page 54

1. Which promise is unsafe?

B. Nothing you say can ever be shared under any circumstances.

Absolute secrecy ignores possible legal, safety, and record-access limits.

2. Which issue is separate from privacy in the room?

B. Portal and billing visibility

A private conversation can still generate records or communications with separate access arrangements.

3. Which legal statement is justified here?

C. Applicable rules must be verified.

The case supplies no jurisdiction and therefore requires checking the actual rules.

4. What should happen before sensitive disclosure?

B. Explain relevant privacy limits

Explaining limits first helps the adolescent understand how information may be handled.

06 Conversation answers

Explaining adolescent confidentiality

E Dialogue gaps

Pages 55-56

1. confidentiality. Turn 2. Confidentiality protects information within applicable limits; it is not absolute secrecy.

2. private consultation. Turn 4. A private consultation does not automatically make subsequent records or messages private.

3. proxy access. Turn 6. Proxy access allows another person specified access and needs actual verification.

4. billing disclosure. Turn 8. Billing disclosure may reveal information separately from the privacy of the consultation.

5. Minor consent. Turn 10. Minor-consent rules vary by jurisdiction and the particular care involved.

6. safety exception. Turn 12. A safety exception depends on applicable duties, not a universal secrecy promise.

7. Mandatory reporting. Turn 14. Mandatory reporting is a legal duty to communicate specified concerns and must be explained according to the actual applicable requirements.

8. preferred contact method. Turn 16. A preferred contact method identifies the desired communication route, which the service must check against its actual privacy and operational arrangements.

9. record-access rule. Turn 18. A record-access rule governs who can see particular information and should be verified before assurances are given about portal or record privacy.

10. supported disclosure. Turn 20. Supported disclosure involves explaining and supporting the appropriate sharing process rather than leaving the young person surprised about how information is handled.

F Transfer conversation

Page 57

1. shared

Teenager: "That phone is shared."

The listed phone is shared rather than known to be private.

2. verified

Clinician: "A private route is not yet verified."

The clinic has not confirmed an appropriate confidential contact route.

3. sent

Teenager: "No sensitive message has been sent."

The scenario explicitly states that no sensitive message has been sent.

4. arrangements

Clinician: "We need to check the contact arrangements."

The actual contact process must be checked before assurances are given.

07 Briefing and language checks

Coordinating care with a school

A Briefing checks / page 58

1. What does the school hold?

D. An older action-plan version

Rivera reports an older plan rather than a verified current version.

2. What must be checked?

B. The current plan and authorized sharing

The case requires verification and appropriate sharing of the clinician-approved plan.

3. What is not permitted by the exercise?

C. Independent treatment changes

No dose, threshold, or independent authority to alter treatment is supplied.

D Language checks / page 63

1. Which question addresses version control?

A. Which dated plan does the school hold?

The date and version identify whether the school holds the appropriate current plan.

2. What is not equivalent to implementation?

C. Sending the document

A sent file may not have been received or made usable by the responsible people.

3. Which instruction is unsupported?

B. Increase the inhaler dose from this exercise.

The exercise provides no dose or authority for an independent treatment change.

4. Why check a field trip separately?

C. The practical setting and arrangements may differ.

Off-site activities may require different practical preparation within the actual approved care arrangements.

07 Conversation answers

Coordinating care with a school

E Dialogue gaps

Pages 64-65

- 1. plan version.** Turn 2. Plan version identifies the actual edition; an available copy may be outdated.
- 2. asthma action plan.** Turn 4. An asthma action plan contains individualized instructions, not an improvised substitute.
- 3. medication authorization.** Turn 6. Medication authorization does not follow automatically from possessing the care document.
- 4. Self-carry permission.** Turn 8. Self-carry permission needs the actual authorization process, not an assumed entitlement.
- 5. receipt confirmation.** Turn 10. Receipt confirmation establishes arrival, not readiness to implement every instruction.
- 6. Implementation readiness.** Turn 12. Implementation readiness requires the actual people and practical arrangements, not just paperwork.
- 7. field-trip arrangement.** Turn 14. A field-trip arrangement addresses the practical health preparation for an off-site activity and should not be assumed identical to the classroom setup.
- 8. emergency action.** Turn 16. Emergency action follows the student's real urgent-care plan and applicable procedures, not an invented threshold from a language exercise.
- 9. activity accommodation.** Turn 18. An activity accommodation supports participation through suitable adjustments and should be coordinated with the actual clinical and school arrangements.
- 10. care coordination.** Turn 20. Care coordination aligns information and responsibilities across the family, school, and clinical team rather than ending when a document is sent.

F Transfer conversation

Page 66

1. received

Nurse: "The current plan has been received."

The school confirms receipt of the verified current document.

2. pending

Clinician: "Staff preparation remains pending."

The scenario says the required preparation is not yet complete.

3. readiness

Nurse: "Receipt does not prove readiness."

A document's arrival is distinct from practical implementation readiness.

4. arrangements

Clinician: "Please confirm the remaining arrangements."

The outstanding staff preparation and practical arrangements require follow-up.

08 Briefing and language checks

Handing over an urgent change

A Briefing checks / page 67

1. What change does Mei report?

B. Much less responsiveness and poor feeding

The supplied concern is a marked change in responsiveness with poor feeding.

2. What is already active?

D. The emergency response

The emergency response is active and must not wait for this exercise.

3. What must remain unknown here?

D. The diagnosis and measured observations

No diagnosis or measurements are supplied, so they cannot be invented.

D Language checks / page 72

1. Which opening conveys the priority?

C. Arlo is much less responsive than usual and feeding poorly.

The opening conveys the reported change without inventing measurements or diagnosis.

2. Which statement is unsupported?

B. Arlo's measured observations are normal.

No measured observations are supplied, so normal cannot be asserted.

3. What should not delay care?

A. Completing the language exercise

Language practice must never postpone the active clinical response.

4. What does read-back accomplish?

C. Checks accurate receipt of key information

Repeating the key information verifies communication, not diagnosis or outcome.

08 Conversation answers

Handing over an urgent change

E Dialogue gaps

Pages 73-74

1. emergency response. Turn 2. The emergency response continues without waiting for completion of this discussion.

2. responsiveness. Turn 4. Responsiveness is the caregiver's report here, not an invented clinical score.

3. observed behavior. Turn 6. Observed behavior identifies what the caregiver witnessed, distinct from clinical measurements.

4. poor feeding. Turn 8. Poor feeding is a reported change, not an established cause.

5. last known usual state. Turn 10. Last known usual state records the most recent observed baseline without delaying care.

6. caregiver concern. Turn 12. Caregiver concern can identify an important change and should be acknowledged without demanding clinical terminology or false precision.

7. handoff priority. Turn 14. Handoff priority identifies the most important information to lead the transfer, here the reported change in responsiveness and feeding.

8. measured observation. Turn 16. A measured observation must come from actual assessment and cannot be called normal when no value or finding has been supplied.

9. receiving clinician. Turn 18. The receiving clinician accepts the relevant information and responsibility and should be explicitly identified rather than assumed.

10. read-back confirmation. Turn 20. Read-back confirmation repeats key information to check accurate receipt while the actual clinical response continues.

F Transfer conversation

Page 75

1. 07:30

Caregiver: "The corrected time is 07:30."

The caregiver explicitly corrects the last observed usual state to 07:30.

2. 08:00

Clinician: "The earlier report of 08:00 is superseded."

The prior time is replaced by the caregiver's corrected account.

3. active

Caregiver: "The emergency response remains active."

Correcting the timeline does not interrupt the ongoing emergency response.

4. unestablished

Clinician: "The diagnosis remains unestablished."

The timing correction does not establish a clinical diagnosis.

08

09 Conversation answers

Preparing for an adult-care transition

E Extended conversation

Pages 76-77

1. transition plan. Turn 2. A transition plan organizes preparation and responsibilities for moving between services rather than treating the referral alone as a completed transfer.

2. condition summary. Turn 4. A condition summary presents the relevant diagnosis and care history in a usable form and must be checked rather than reconstructed from uncertain memory.

3. receiving service. Turn 8. The receiving service is the team expected to accept the next stage of care and its actual acceptance and appointment arrangements need confirmation.

4. communication arrangements. Turn 12. Communication arrangements specify contacts, permissions, and access routes and must be checked under the relevant service and legal rules rather than a universal age assumption.

5. readiness discussion. Turn 16. A readiness discussion explores the person's understanding, practical skills, and support needs rather than assuming independence from age alone.

6. confirmed transfer. Turn 20. A confirmed transfer establishes that the receiving service has accepted the relevant care responsibilities, rather than merely receiving a referral request.

F Transfer exchange

Page 78

1. received

Kai: "The referral has been received."

The adult service confirms receipt of the referral request.

2. booked

Pediatrician: "A visit is not yet booked."

Receipt does not establish a scheduled appointment in this scenario.

3. clarified

Kai: "Interim responsibility needs to be clarified."

The teams must identify who is responsible before the transfer is complete.

4. letter

Pediatrician: "The transfer is not simply a sent letter."

A sent referral document alone does not establish a completed care transfer.

10 Conversation answers

A school report with limited sharing permission

E Extended conversation

Pages 79-80

1. functional need. Turn 4. A functional need describes the relevant practical difficulty or support requirement and is distinct from disclosing an entire clinical history.

2. information-sharing permission. Turn 6. Information-sharing permission defines what may be disclosed, for what purpose, and to whom under the applicable rules and authorization.

3. designated contact. Turn 10. A designated contact is the named person responsible for the relevant communication and helps avoid uncontrolled distribution or contradictory messages.

4. unsupported statement. Turn 14. An unsupported statement lacks the relevant evidential basis and should not be added merely because an administrative form requests it.

5. report scope. Turn 16. Report scope identifies the subjects and limits covered by the actual assessment and authorization rather than implying a comprehensive opinion on every school issue.

6. purpose-limited disclosure. Turn 20. A purpose-limited disclosure shares information appropriate to the authorized purpose rather than releasing unrelated history without justification.

F Transfer exchange

Page 81

1. named

Caregiver: "That recipient was not named."

The new staff member was not included in the original permission.

2. authorization

Clinician: "We need to check authorization."

Sharing with the new recipient requires verification under the applicable process.

3. sent

Caregiver: "The report has not been sent to them."

The scenario states that no disclosure to the new recipient has occurred.

4. unlimited

Clinician: "The original permission was not unlimited."

A limited information-sharing permission does not automatically cover every later recipient.

10

11 Conversation answers

Reconciling two feeding accounts

E Extended conversation

Pages 82-83

1. observation window. Turn 4. An observation window is the period covered by a report and must be clarified before two accounts are treated as describing the same events.

2. measurement unit. Turn 6. A measurement unit specifies how an amount is expressed and must be clear before quantities from different reports are compared.

3. Offered volume. Turn 8. Offered volume is the amount made available and is not automatically the amount actually taken during the feed.

4. estimated amount. Turn 10. An estimated amount is an approximation and should remain distinguishable from a measured quantity in the feeding history.

5. source-specific summary. Turn 16. A source-specific summary keeps each caregiver's period and observations identifiable rather than blending them into an apparently precise but unsupported total.

6. history reconciliation. Turn 20. History reconciliation aligns accounts by checking sources, timing, units, and uncertainty rather than choosing a winner or manufacturing a combined number.

F Transfer exchange

Page 84

1. offered

Caregiver: "My figure describes what was offered."

One report concerns the amount made available during feeding.

2. taken

Clinician: "The other describes what was taken."

The second report concerns the amount actually taken rather than offered.

3. same

Caregiver: "The units are the same."

The scenario states that the units match even though the quantities differ.

4. calculated

Clinician: "No combined total has been calculated."

The exercise does not combine the different quantities into an intake total.

Find the move you need

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

01 Hearing the child and caregiver

Invite the child: Leo, tell me what happens in your own words.

Ask about impact: What becomes difficult when it happens?

Summarize: Tell me which part of my summary needs correcting.

Full phrase set: pages 7-8 | Dialogue: pages 10-11

02 Explaining a growth trajectory

Check the interpretation: What did the number suggest to you?

Shift to the pattern: Let us look at the measurements over time.

Explain the review: The pattern and the child's circumstances belong together.

Full phrase set: pages 16-17 | Dialogue: pages 19-20

03 Discussing developmental screening

Name the role: This questionnaire identifies concerns that need a closer look.

Include languages: Tell me which languages Nia hears and uses.

Avoid a false promise: I cannot promise the assessment's conclusion in advance.

Full phrase set: pages 25-26 | Dialogue: pages 28-29

04 Responding to vaccine questions

Find the concern: What happened after the previous vaccination?

Separate cause and timing: Happening afterward does not by itself establish the cause.

Close with understanding: Please tell me how you understand the next step.

Full phrase set: pages 34-35 | Dialogue: pages 37-38

Keep the conversation productive

Use these as precise language models. Replace details only when you know the actual facts, authority, and agreed conditions.

05 Checking liquid-medicine instructions

Separate the units: Milligrams and milliliters measure different things.

Avoid substitution: Do not assume another bottle has the same concentration.

Own the explanation: Let me explain the units more clearly.

Full phrase set: pages 43-44 | Dialogue: pages 46-47

06 Explaining adolescent confidentiality

Avoid absolute secrecy: I cannot promise that every circumstance is completely private.

Check access: I need to verify who can see this part of the record.

Support sharing: When sharing is needed, I will explain the process and support you.

Full phrase set: pages 52-53 | Dialogue: pages 55-56

07 Coordinating care with a school

Verify the version: Which dated version does the school currently hold?

Avoid improvisation: Please do not substitute an unverified dose or instruction.

Close the loop: We need both the current document and confirmed responsibilities.

Full phrase set: pages 61-62 | Dialogue: pages 64-65

08 Handing over an urgent change

Lead with the change: You are reporting a clear change from Arlo's usual behavior.

Preserve the source: I will pass on exactly what you observed.

Check the message: Please repeat the key change so we can confirm accurate receipt.

Full phrase set: pages 70-71 | Dialogue: pages 73-74

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

absolute risk	communicate absolute risk	33
active ingredient	identify the active ingredient	41
activity accommodation	discuss activity accommodations	60
acute change	communicate an acute change	68
adaptive skill	assess adaptive skills	23
administration interval	confirm the administration interval	41
administration record	check the administration record	33
adverse event	document an adverse event	32
adverse reaction	distinguish an adverse reaction	32
appetite change	report an appetite change	5
as-needed instruction	clarify an as-needed instruction	41
associated symptom	ask about associated symptoms	5
asthma action plan	verify the asthma action plan	59
baseline behavior	establish baseline behavior	5
billing disclosure	explain possible billing disclosure	50
body mass index	explain body mass index	14
breakthrough infection	discuss breakthrough infection	33
calibration mark	read the calibration marks	41
care coordination	strengthen care coordination	60
caregiver account	attribute the caregiver account	5
caregiver concern	acknowledge caregiver concern	68
caregiver demonstration	invite a caregiver demonstration	42
caregiver questionnaire	review a caregiver questionnaire	23
catch-up schedule	verify a catch-up schedule	33

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

causal relationship assess a causal relationship	32
centile crossing review centile crossing	15
child account invite the child account	5
clinical deterioration communicate suspected clinical deterioration	69
clinical handoff confirm the clinical handoff	6
clinical handoff confirm the clinical handoff	15
clinical handoff confirm the clinical handoff	24
clinical handoff confirm the clinical handoff	33
clinical handoff confirm the clinical handoff	42
clinical handoff confirm the clinical handoff	51
clinical handoff confirm the clinical handoff	60
clinical handoff confirm the clinical handoff	69
concentration verify the concentration	41
confidential contact arrange a confidential contact	51
confidentiality explain confidentiality	50
consent capacity assess consent capacity	50
contextual difference explore contextual differences	6
contraindication verify a contraindication	32
controller medicine explain controller medicine	59
corrected age clarify corrected age	15
developmental language use developmental language	6
developmental milestone discuss developmental milestones	23
developmental screening explain developmental screening	23
developmental surveillance conduct developmental surveillance	23

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

diagnostic evaluation arrange a diagnostic evaluation	23
direct observation distinguish direct observation	6
disease protection explain disease protection	33
dispensing pharmacy contact the dispensing pharmacy	42
documented plan review the documented plan	6
documented plan review the documented plan	15
documented plan review the documented plan	24
documented plan review the documented plan	33
documented plan review the documented plan	42
documented plan review the documented plan	51
documented plan review the documented plan	60
documented plan review the documented plan	69
dose amount confirm the dose amount	41
dose volume verify the dose volume	41
duplicate ingredient check for duplicate ingredients	41
early intervention discuss early-intervention services	24
emergency action confirm emergency actions	59
emergency response confirm the emergency response	69
episode duration establish episode duration	5
exercise-related symptom report exercise-related symptoms	59
expected reaction explain an expected reaction	32
expressive language describe expressive language	23
false-positive screen explain a possible false-positive screen	24
familial pattern consider the familial pattern	15

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

febrile illness	assess a febrile illness	68
field-trip arrangement	confirm field-trip arrangements	60
fine motor skill	describe fine motor skills	23
functional impact	assess functional impact	5
gross motor skill	review gross motor skills	23
growth faltering	assess suspected growth faltering	15
growth trajectory	review the growth trajectory	14
growth velocity	assess growth velocity	14
guardian authority	clarify guardian authority	50
handoff priority	state the handoff priority	69
head circumference	record head circumference	14
hearing assessment	arrange a hearing assessment	24
hydration assessment	perform a hydration assessment	68
immunization	discuss immunization	32
implementation readiness	check implementation readiness	60
information sharing	discuss information sharing	50
informed choice	support informed choice	32
inhaler technique	review inhaler technique	59
label discrepancy	resolve a label discrepancy	42
last known usual state	clarify the last known usual state	68
length-for-age	review length-for-age	14
longitudinal assessment	conduct a longitudinal assessment	15
mandatory reporting	clarify mandatory-reporting duties	50
maximum daily amount	verify the maximum daily amount	42

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

measured observation	relay measured observations	68
measurement technique	verify measurement technique	14
measuring device	match the measuring device	42
medication authorization	verify medication authorization	59
medication reconciliation	complete medication reconciliation	42
milligram	distinguish milligrams	41
milliliter	measure in milliliters	41
minimum necessary information	identify necessary information	50
minor consent	verify minor-consent rules	50
multilingual exposure	describe multilingual exposure	24
nutritional history	obtain a nutritional history	15
observed behavior	describe observed behavior	68
oral syringe	select the appropriate oral syringe	41
patient preference	elicit the patient preference	6
patient preference	elicit the patient preference	15
patient preference	elicit the patient preference	24
patient preference	elicit the patient preference	33
patient preference	elicit the patient preference	42
patient preference	elicit the patient preference	51
patient preference	elicit the patient preference	60
patient preference	elicit the patient preference	69
percentile	explain a percentile	14
plan version	verify the plan version	59
plotting error	check for a plotting error	14

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

poor feeding	report poor feeding	68
portal visibility	verify portal visibility	50
precaution	review a relevant precaution	32
preferred contact method	confirm the preferred contact method	51
prescription label	check the prescription label	41
presenting concern	clarify the presenting concern	5
privacy limitation	explain privacy limitations	51
private consultation	offer a private consultation	50
proxy access	check proxy access	50
qualified interpreter	request a qualified interpreter	6
qualified interpreter	request a qualified interpreter	15
qualified interpreter	request a qualified interpreter	24
qualified interpreter	request a qualified interpreter	33
qualified interpreter	request a qualified interpreter	42
qualified interpreter	request a qualified interpreter	51
qualified interpreter	request a qualified interpreter	60
qualified interpreter	request a qualified interpreter	69
rapport	build rapport	6
read-back confirmation	obtain read-back confirmation	69
recalled event	clarify a recalled event	6
receipt confirmation	obtain receipt confirmation	60
receiving clinician	identify the receiving clinician	69
receptive language	assess receptive language	23
record-access rule	verify record-access rules	51

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

reference population	identify the reference population	14
referral status	check referral status	24
reliever medicine	clarify reliever instructions	59
responsiveness	describe reduced responsiveness	68
safeguarding concern	address a safeguarding concern	51
safety exception	explain a safety exception	50
safety monitoring	explain safety monitoring	33
school attendance	clarify school attendance	5
school contact	identify the school contact	60
school health plan	update the school health plan	59
screen-positive result	explain a screen-positive result	23
self-carry permission	check self-carry permission	59
serial measurements	review serial measurements	14
skill regression	report skill regression	24
sleep pattern	describe the sleep pattern	5
social communication	explore social communication	23
source attribution	preserve source attribution	6
source attribution	preserve source attribution	15
source attribution	preserve source attribution	24
source attribution	preserve source attribution	33
source attribution	preserve source attribution	42
source attribution	preserve source attribution	51
source attribution	preserve source attribution	60
source attribution	preserve source attribution	69

Find a term and its context

The page number leads to the definition and a typical word combination. Repeated terms have separate entries when they serve different lesson contexts.

spacer device	check the spacer device	59
stature-for-age	assess stature-for-age	14
summary check	perform a summary check	6
supported disclosure	arrange supported disclosure	51
symptom frequency	describe symptom frequency	5
symptom onset	clarify symptom onset	5
teach-back	use teach-back without blame	6
teach-back	use teach-back without blame	15
teach-back	use teach-back without blame	24
teach-back	use teach-back without blame	33
teach-back	use teach-back without blame	42
teach-back	use teach-back without blame	51
teach-back	use teach-back without blame	60
teach-back	use teach-back without blame	69
temporal association	explain temporal association	32
trigger exposure	describe trigger exposure	59
urgent escalation	initiate urgent escalation	69
urine output	clarify urine output	68
usual pattern	establish the usual pattern	68
vaccination history	verify vaccination history	32
vaccine hesitancy	explore vaccine hesitancy	32
vaccine recommendation	review the vaccine recommendation	32
weight-for-age	plot weight-for-age	14
work of breathing	assess work of breathing	68

Original cases. Clear boundaries.

These are original fictional teaching conversations, not transcripts of real workers. Names, figures, organizations, and incidents are invented. Sources support terminology and communication aims, not the fictional results or an endorsement of this book.

[1] AAP HealthyChildren. Understanding Growth Charts.

[Open the original source](#)

Context for explaining measurements and patterns without treating percentiles as grades. Checked 10 October 2026.

[2] AAP HealthyChildren. Assessing Developmental Delays.

[Open the original source](#)

Background for distinguishing screening concerns from a diagnostic assessment. Checked 10 October 2026.

[3] AAP HealthyChildren. How to Use Liquid Medicines for Children.

[Open the original source](#)

Supports precise liquid-measurement language and appropriate measuring tools; no doses are supplied here. Checked 10 October 2026.

[4] AAP HealthyChildren. Information for Teens: Privacy.

[Open the original source](#)

Context for adolescent privacy conversations; actual legal and record-access rules must be checked locally. Checked 10 October 2026.

Phil Stilwell

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